



Medical Council

Medical Workforce Intelligence Report 2023



Comhairle na nDochtúirí Leighis
Medical Council

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This report was prepared by the Research and Regulatory Data Insights Team at the Medical Council. We wish to extend special thanks to the Registration Team of the Medical Council's executive for their support. This research report could not have been produced without the 24,794 doctors who retained registration with the Medical Council in 2023. We hope that through their contributions, this report can help further strengthen and develop a supported and resourced medical workforce that provides quality and safe healthcare in Ireland.

Executive Summary

This Workforce Intelligence Report analyses and presents data provided by doctors who completed the Medical Council's annual retention process in 2023. Maintaining the Medical Register ('the Register') is fundamental to the Medical Council's remit of protecting the public, and data held on the Register provides valid and robust insight into the medical workforce in Ireland. This report emphasises the clinically active medical workforce in Ireland, presenting quantitative analysis of demographics and divisional status, and details on employment and practise such as self-reported role, medical discipline and speciality, and working hours.

The data outlined in this report point to a number of important observations including challenges facing Non-Consultant Hospital Doctors (NCHDs) who comprise a critical component of the medical workforce and the need to improve the NCHD experience and work life balance. International medical graduates comprise the majority of NCHDs not in training. Work conditions, training and progression opportunities urgently need to be improved and supported to ensure they retain in the health service and continue to meet the demanding challenges of their daily work. The data highlights workforce pressure points across cohorts of specialities, including General Practice, discipline of Medicine, Surgery, and Hospital Consultants, with a substantial proportion of doctors close to retirement age and working in contravention of the European Working Times Directive (EWTD).

The data contained within this report should be used to inform recruitment and retention strategies, medical education and training development and health policy in Ireland. This robust data, combined with employment data, can support effective planning for a strong and sustainable medical workforce. It is currently being used by the Health Service Executive's National Doctors Training and Planning (HSE NDTP) and the Department of Health (DOH) to inform the Health and Social Care Workforce Planning Strategy and Action Plan.

The Council is committed to collaborating with stakeholders to share and support data and evidence that facilitate medical workforce planning. Effective and strategic workforce planning can be achieved through concerted collaborative working amongst policymakers, educators, regulators, planners and employers, including the DOH, Health Service Executive (HSE), National Taskforce on the NCHD Workforce, Forum of Irish Postgraduate Medical Training Bodies and Medical Schools.

Workforce Intelligence Report 2023 in Numbers

19,328 Clinically Active Doctors Reporting to be Working in Ireland

 **10,251** (53.0%)
Males
 **9,077** (47.0%)
Females



Mean age **43.7**

Just over **one in five**
doctors aged **55**
and above



 **11,709** (60.6%) with Irish qualifications
 **2,661** (13.7%) with EU or UK qualifications
 **4,958** (25.7%) with qualifications outside of Ireland, the EU or the UK

Top 3 Countries Of Qualification for International Graduates:

Pakistan: **1,851** (24.3%)
Sudan: **1,060** (13.9%)
United Kingdom: **709** (9.3%)

Divisions of the Register:

 Specialist: **9,700** (50.2%)
 General: **6,369** (33.0%)
 Trainee Specialist: **3,105** (16.1%)
 Supervised: **154** (0.8%)

Self-reported roles:



Hospital Consultant: **4,889** (25.6%)
Other Consultant
or Specialist: **666** (3.5%)
General Practitioner: **4,525** (23.7%)
NCHD in training: **4,001** (21.0%)
NCHD not in training: **4,233** (22.2%)

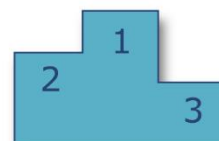
Working hours:

5,066 (26.6%) working
more than 48 hours per week
6,202 (32.6%) working
40 to 48 hours per week



Doctors in Top 3 specialities:

General Practice: **4,990** (25.8%)
General (Internal) Medicine: **1,686** (8.7%)
Anaesthesiology: **1,265** (6.5%)



1. Key Observations

Analysis of workforce registration data highlight key observations and insights that can inform strategic and integrated medical workforce planning:

Non-Consultant Hospital Doctor (NCHD) and International Medical Graduate (IMG) Workforce

NCHD Challenges

NCHDs are a critical part of the healthcare workforce in Ireland, and may be employed in,

- Posts recognised for national specialist training, including Interns, Basic Specialist Training (BST) and Higher Specialist Training (HST), combining formal training with service delivery,
- Posts included in the International Medical Graduate Training Initiative (IMGTI),
- Posts not recognised for training, the purpose of which are service delivery oriented and carried out as part of a medical team,
- Post-Certificate of Satisfactory Completion of Specialist Training (CSCST) fellowship posts recognised by an accredited postgraduate training body.

Safe and timely delivery of healthcare is dependent on NCHDs who need to be supported with appropriate education and training. In 2023, 8,234 doctors self-reported working as NCHDs. Just under half of NCHDs were in training (48.6%), while the remainder were not in training (51.4%). NCHDs work under the supervision of a consultant doctor, however, there is a significant shortage in the number of approved training posts and precarious career training pathways for NCHDs not in training who are filling growing gaps in the healthcare system. Ireland has an excess of NCHDs not in training, relative to specialists, as highlighted in the NDTP's recent medical workforce reports (National Doctors Training & Planning, Medical Workforce Analysis Report, 2023-24).

The majority of NCHDs not in training are doctors who have trained overseas, and many have not been able to access specialist training in Ireland, a pattern that has remained unchanged in recent years. Many are employed in Model 2 and Model 3 hospitals (NDTP, 2022), which have high proportions of NCHDs overall and are heavily reliant on NCHDs not on a training scheme.

Overall, NCHDs are more likely to work excessive hours compared to other doctor cohorts. Our data indicate that close to half of NCHDs in training and almost 30% of NCHDs not in training were working more than 48 hours per week.

Reliance on IMGs

Ireland is highly dependent on IMGs, having the fourth highest proportion of foreign educated doctors within OECD member states (Hynes and O'Connor, 2023). In 2023, our data indicates that just under 40% of the clinically active workforce in Ireland have an international qualification (a basic medical qualification obtained abroad). The proportion of doctors qualified in the EU or UK has remained stable at 13.7% when comparing 2023 and 2022, while the proportion of doctors qualified outside of Ireland, the EU and UK has increased by 2.4% (25.7% vs 23.3%).

Research indicates that internationally trained doctors report entering posts in Irish hospitals with the expectation of career progression or accessing specialist training. Lack of progression opportunities, fear of deskilling, quality of training, excessive work hours and poor working conditions prompt many to actively seek to migrate or return to their home country within a few years of arriving in Ireland (Heffron and Socha-Dietrich, 2019). The National Taskforce on the NCHD Workforce report recognises that newly arrived IMGs do not receive specific induction and support on the Irish healthcare service, and training/induction for IMGs is essential for both patient care and for doctor wellbeing. The Interim Recommendations Report of the Taskforce, published in April 2023, recommended that IMGs new to the Irish health service require enhanced and protected induction, bespoke to the Irish health service. Specific and measurable implementation considerations were included as part of this, and the Final Report of the Taskforce highlights good progress being made on this recommendation to date.

Furthermore, when looking at the proportion of international medical graduates by division this proportion varies. Over 70% of doctors on the Specialist Division and nearly 80% of doctors on the Trainee Specialist Division have Irish qualifications, while only 34% of doctors on the General Division have Irish qualifications. This indicates that the numbers of international doctors are increasing, and that the majority of these doctors are on the General Division. The General Division was created as a transition for doctors progressing from internship to the Trainee Specialist Division, and as such does not offer appropriate supervision and specialist training. This can potentially have implications for patient care and doctor wellbeing.

Workforce Specialties and Challenges

General Practice

General Practitioners (GPs) are often the first point of contact for patients and General Practice is a crucial part of a functional healthcare system in Ireland. Our data indicate that a substantial proportion of GPs are close to retirement age and that women constitute a high proportion of the GP workforce

at just over 50%. These workforce demographics are important for informing future workforce planning, training and education needs and ensuring delivery of timely and high-quality patient care.

New data gathered by the Medical Council for the first time on GP work patterns show a breakdown in relation to numbers of GP principals/partners, single-handed practices, locum etc. The gender breakdown of those working as a principal or partner is closely split, however other roles such as single-handed GP, locum GPs and out-of-hours GPs are male dominant while GP assistant and sessional GP are female dominant.

Increasing pressure on GPs is a result of a growing and ageing population, increased patient multimorbidity, GP retirement, poor morale and burnout (Collins and O’Riordan, 2024). There is an urgent need for effective action to tackle Ireland's shortage of GPs, which can potentially impact the core elements of general practice, namely, continuity of care, the doctor-patient relationship and person-centred care.

Medicine

Within the discipline of medicine, e.g., Cardiology, General (Internal) Medicine, the division with the highest proportion of doctors was the General Division (44.6%). Irish qualifications were held by just under 60% of this cohort.

Of those doctors within this category who are on the Specialist Division a relatively high proportion are approaching retirement age, with almost 30% of the cohort aged 55 and over. Within this cohort on the Specialist Division, Cardiology, Nephrology and Oncology have the most doctors working in excess of 48 hours per week. Female doctors are more strongly represented than males in some medicine specialities, such as Dermatology and Palliative Medicine, while male doctors constitute a notably higher proportion in Cardiology, Gastroenterology, General (Internal) Medicine and Respiratory Medicine.

Surgery

Within the medical discipline of surgery, comprising 11 specialities and 2,320 doctors, just over three in four were male (76.3%). Almost one in five (19.4%) were aged 55 or over. Of those doctors registered on the Specialist Division of this discipline and working in General Surgery, over 40% are aged 55 and over, rising to 50% among those working in Paediatric Surgery. This age profile is important from the perspective of anticipating retirements. Working in excess of 48 hours per week

is a strong feature among this cohort of doctors, especially among those working in Paediatric Surgery, Vascular Surgery, Cardiothoracic Surgery, Oral and Maxillo Facial Surgery and Neurosurgery. At least half, of all doctors working in these areas of surgery were working in excess of 48 hours per week.

Hospital Consultants

Among hospital consultants, those working in both publicly and privately funded services were most likely to work in excess of 48 hours per week. Those working in private services had the greatest percentage of doctors aged 55 and over, although this is a feature across hospital consultants in all service categories. Recent NDTP medical workforce data indicate that many consultants exit the public health service prior to the age of 65. For consultants over the age of 55 years, the average age at which they leave the public health service is 62. This is important information for workforce planning.

Sufficient numbers of consultants need to be trained and retained in order to have suitably trained applicants for new and replacement consultant posts. Consultant posts are not being adequately filled and this impacts on supervision and training for trainees and junior doctors. From the Medical Council perspective, if doctors do not receive appropriate education and training, patient and professional safety issues arise.

2. Background

The Medical Council is the regulatory body for doctors in Ireland. It has a statutory role in protecting the public by promoting the highest professional standards amongst doctors practising in the Republic of Ireland.

The Medical Register is a valid and complete list of doctors who are permitted under Irish law to practise medicine in the State. The cornerstone of the Medical Council's work in protecting the public is establishing and maintaining a Register of doctors. Under Irish law, no one can practise medicine in Ireland unless they are registered as a doctor with the Medical Council. Data held on the Register provides valid and robust insight into the medical workforce in Ireland.

Doctors register in one of six divisions of the Register, depending on the training they have completed, or are currently undertaking. The six divisions of the Register are:

- **Specialist Registration:** Doctors may practise independently, without supervision and may represent themselves as specialists.
- **Trainee Specialist Registration:** Doctors are on recognised training programmes and practise solely within the confines of posts allocated by the HSE in conjunction with the postgraduate training bodies (PGTBs).
- **General Registration:** Doctors may practise independently, without supervision but may not represent themselves as specialists.
- **Supervised Registration:** Doctors who have been offered a post that has been approved by the HSE, which has specific supervisory arrangements.
- **Internship Registration:** Allows a doctor to carry out internship training in a hospital recognised by the Medical Council. These posts are allocated by the HSE.
- **Visiting EEA Practitioners Registration:** Doctors are European Union citizens who are fully established to practise medicine in another European Union member state. These doctors may practise medicine in Ireland on a temporary basis without having to take out General or Specialist Registration.

3. Methods

3.1 Data Sources

Data contained within this report comes from the annual retention process that doctors undergo with the Medical Council. All doctors practising medicine in Ireland are obligated to renew their registration with the Medical Council annually. Failure to complete this process can ultimately result in a doctor's name being removed from the Register, following a warning and notification of potential removal. All doctors who are active on the Register as of the 30th April each year are invited to undergo retention. The annual retention process is available through each doctor's personal online registration account. Doctors must submit a completed annual retention application form (ARAF) along with the relevant fee.

The ARAF has several compulsory questions where doctors must make declarations about their current position. Since 2012, the Medical Council has included a set of questions to collect and analyse data on registered doctors' work practices to inform strategic workforce planning, regulatory operations and development. These questions are voluntary but have a high response rate. These data have been analysed against basic information about doctors' age, gender, country of qualification and divisional credentials that are housed on the Medical Councils data management system.

3.2 Filtering Active Workforce Data

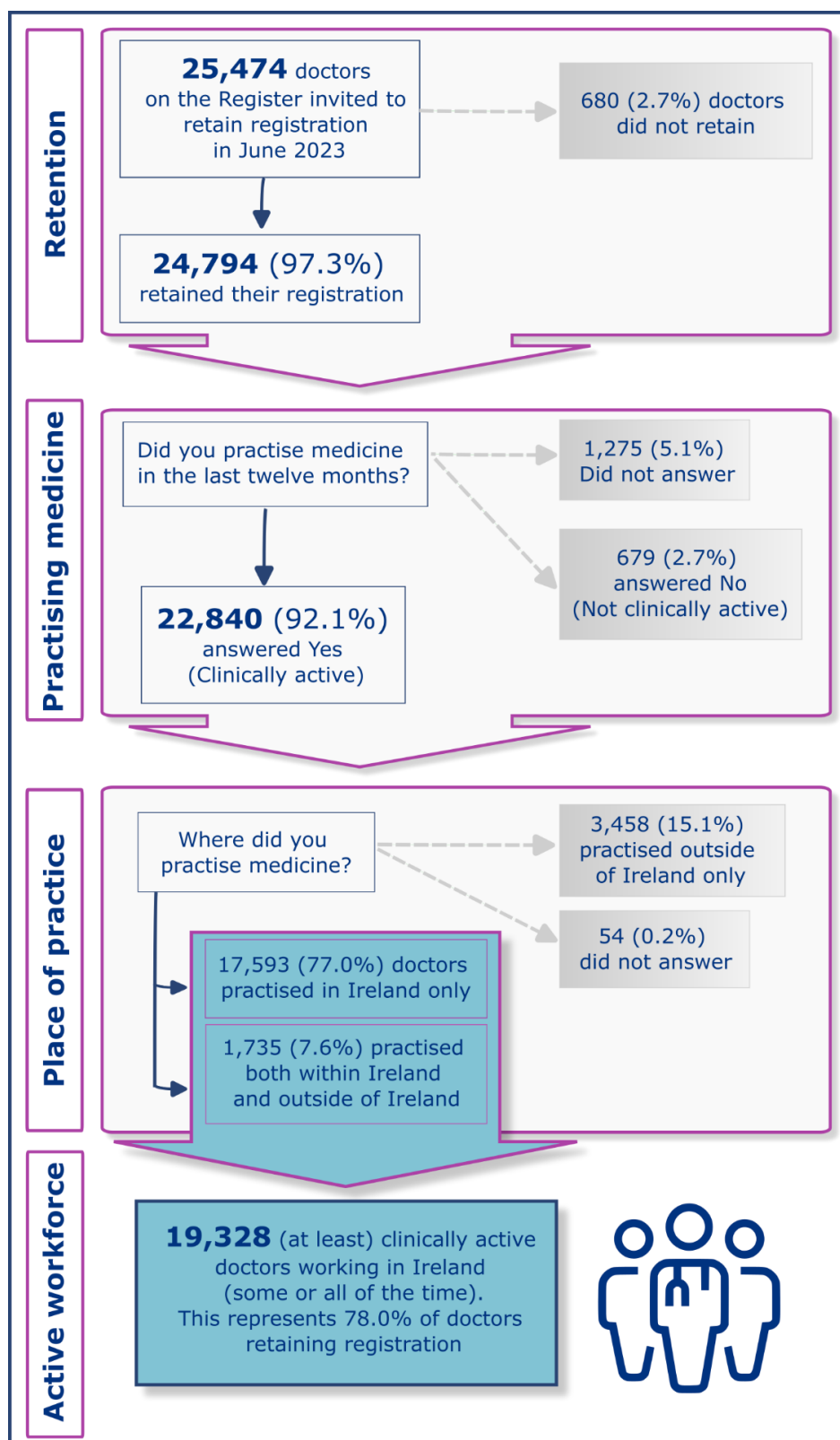
The process of filtering data and selecting the active workforce cohort for analysis consists of a number of steps.

In June 2023, 25,474 doctors on the Register of Medical Practitioners in Ireland were invited to retain their registration with the Medical Council. Of these doctors, 24,794 (97.3%) chose to retain their registration. Doctors who retain registration are asked if they have practised medicine in the previous 12 months. Notably, this question is retrospective and while it is likely that if a doctor practised in the previous 12 months, they would still be clinically active during the retention period, a small number may have subsequently stopped practising medicine. Furthermore, it is not compulsory for doctors to answer this question and as such the figures may be very slightly under the true number. In 2023, 5.1% ($n = 1,275$) of doctors retaining did not provide an answer.

Those who answer yes are then asked a follow-up question on their place of practise (in Republic of Ireland only, both within and outside the Republic of Ireland or outside the Republic of Ireland only). The active medical workforce consists of those **clinically active** doctors who work in the Republic of Ireland (all of the time) and those working both within and outside the country (working in Ireland

some of the time). In 2023, this cohort consisted of 19,328 doctors, which represent 78.0% of all doctors retaining registration. Figure 1 illustrates this process in more detail.

Figure 1. *Filtering Workforce Data*



This data provides a comprehensive overview of the clinically active workforce in Ireland, however, certain cohorts are not captured.

- Doctors who have just completed their 'internship' year, are not required to complete the annual retention process but rather they apply to the Medical Council to transfer registration.
- Doctors who were registered for the first time or who restored registration after April 30th 2023 are not captured in the retention data as they are not required to renew their registration during this period. This includes all doctors commencing their internship.
- Doctors who hold Visiting EEA registration are similarly not required to apply to retain registration with the Medical Council as this is time limited.

Additionally, it is important to reiterate that given the fact that 1,275 doctors did not answer the question on whether they practised medicine in the last 12 months or not, the real number of clinically active doctors working in Ireland in 2023 could have been between 19,328 and 20,603.

4. Clinically Active Doctors in Ireland

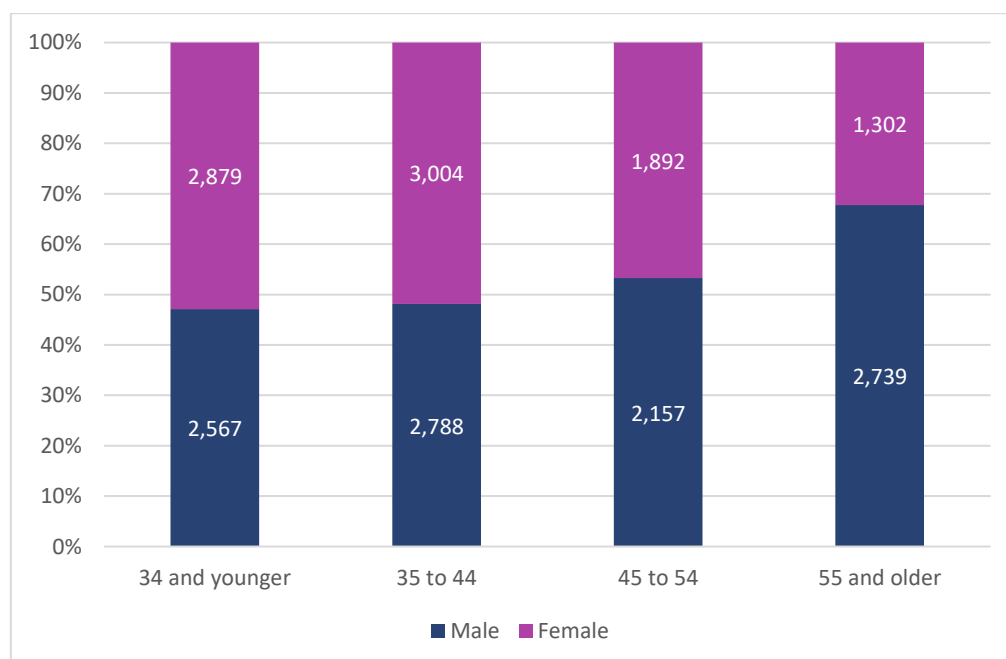
The sections which follow focus on doctors reporting to be clinically active and working in Ireland, all or some of the time ($N = 19,328$).

4.1 Demographics and Registration Division

The gender breakdown of clinically active doctors in Ireland consisted of 10,251 (53.0%) males and 9,077 (47.0%) females. The mean age of clinically active doctors in Ireland was 43.7 (SD = 12.1) years.

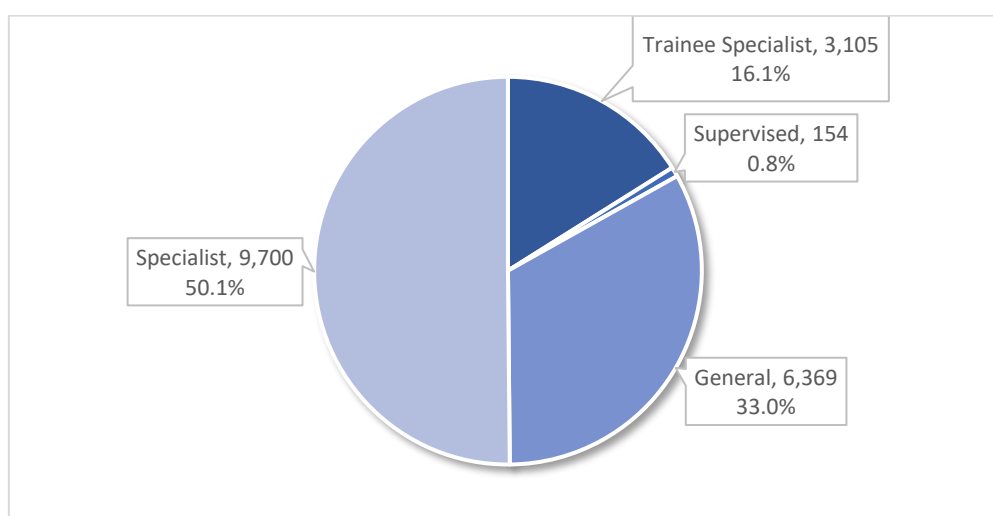
Just over one in five doctors ($n = 4,041$, 20.9%) were aged 55 and above, and therefore approaching the typical retirement age in the next 10 years. The distribution of females and males tended to be split more evenly in younger cohorts, with slightly more females than males aged 34 and younger and 35-44 years. Inversely, there were slightly more males than females aged 45 to 54, and the majority of older doctors, aged 55 and older were male.

Figure 2. Age Groups by Gender



The division with the largest proportion of clinically active doctors was the Specialist Division, with 9,700 members (50.2%), followed by the General Division with 6,369 members (33.0%) and the Trainee Specialist Division with 3,105 members (16.1%) (Figure 3).

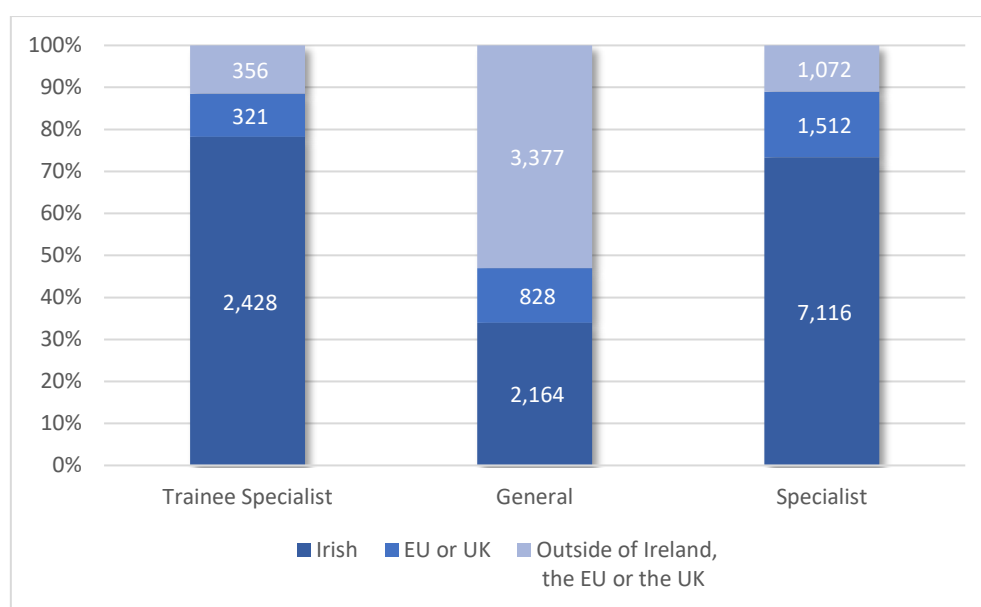
Figure 3. Divisions of Register



In relation to basic medical qualification categories (BMQ), 11,709 (60.6%) clinically active doctors had obtained their qualification in Ireland, 2,661 (13.7%) in the EU or UK and 4,958 (25.7%) outside of Ireland, the EU or the UK.

As can be observed from Figure 4, the majority of those enrolled in the Trainee Specialist Division ($n = 2,428$, 78.2%) and the Specialist Division ($n = 7,116$ 73.4%) had obtained their qualification in Ireland, while over half of doctors on the General Division had obtained their qualification outside of Ireland, the EU or the UK ($n = 3,377$, 53.0%) and a further 13.0% ($n = 828$) had obtained their qualification from the EU or the UK.

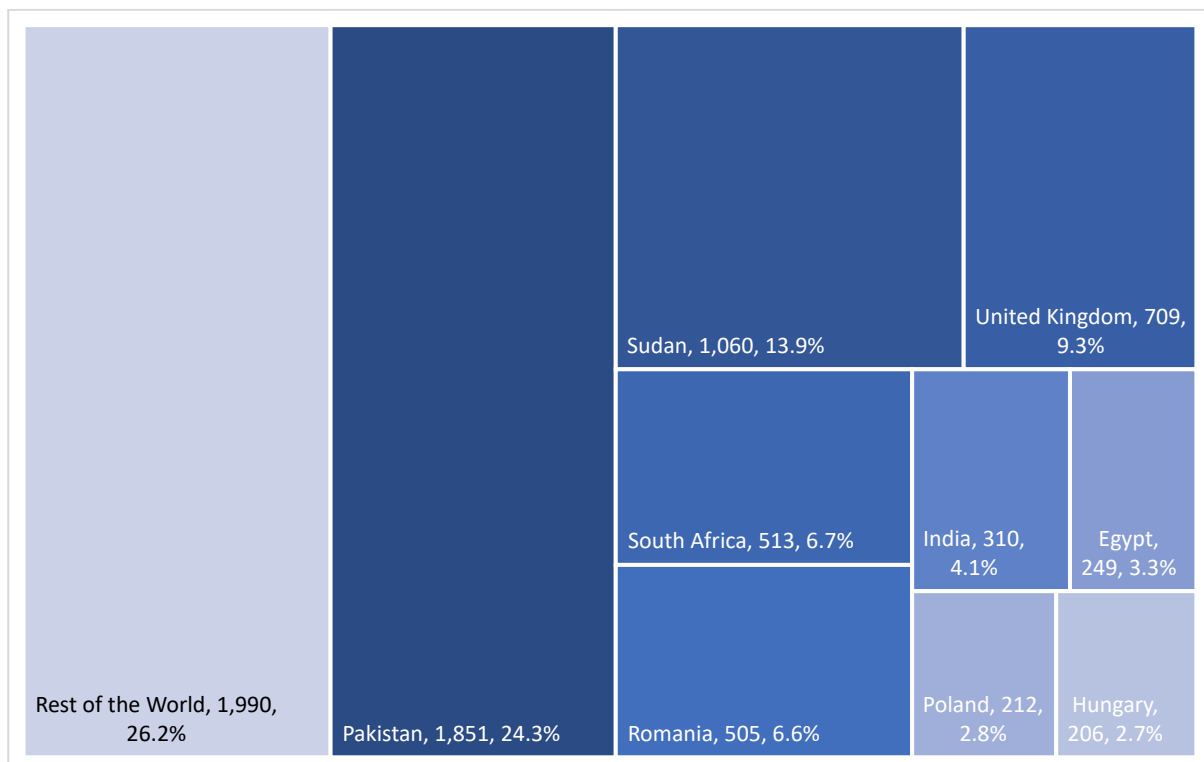
Figure 4. Division by Qualification Category



Note. Doctors on the supervised division are excluded from the above graph.

Qualification category was further condensed into those who received their BMQ from Ireland (Irish Graduates) and those who received their BMQ outside of Ireland (International Graduates). Considering those with a BMQ from outside of Ireland, the three most common countries where international graduates received their medical degrees were Pakistan, Sudan, and the United Kingdom, accounting for 47.5% ($n = 3,620$) of doctors in this cohort. More details on country of qualification can be observed in Figure 5.

Figure 5. Country of Qualification for International Graduates



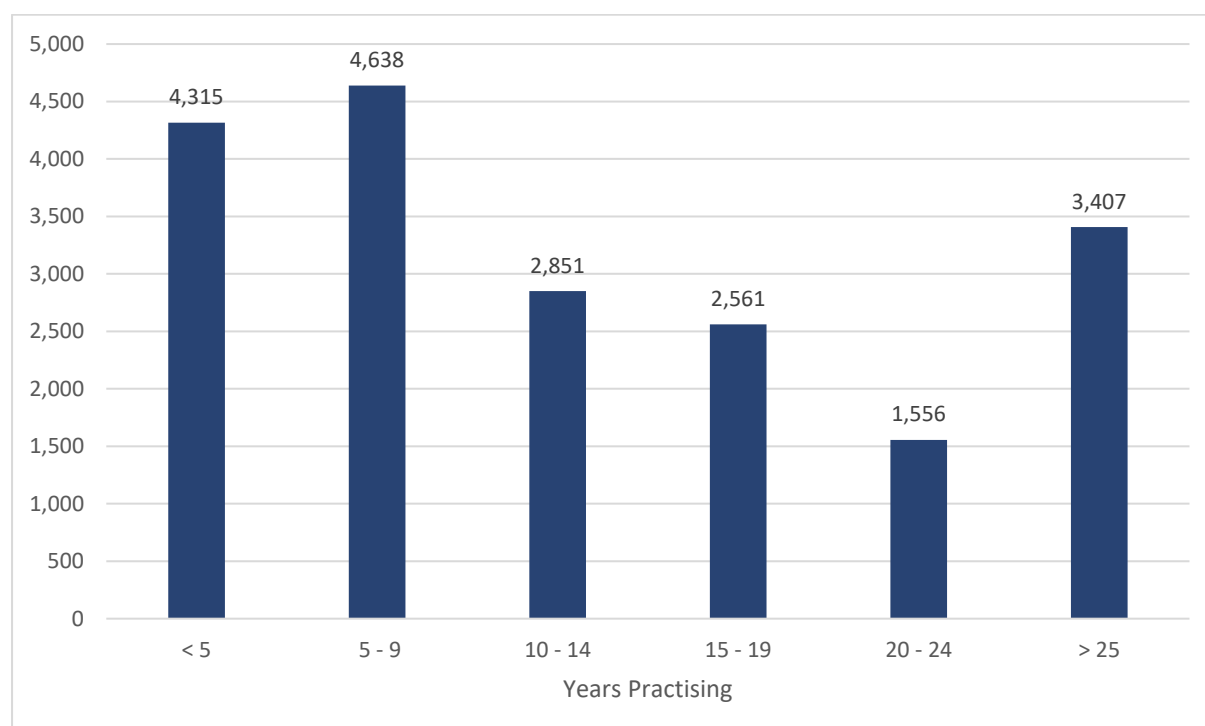
Note. This information was not available for 14 doctors and these are excluded from this figure.

4.2 Employment and Practise

Years of Practise

Doctors' years of practise were calculated from their date of first registration with the Medical Council. This estimate does not exclude any time doctors might have temporarily withdrawn from the Register, or practised outside the State, but offers a rough approximation of how many years of practise in Ireland they may have. Figure 6 shows a breakdown of years of practise.

Figure 6. Doctors' Years of Practise



Intention to Practise

In 2023, doctors were asked if they intend to be in medical practise in Ireland at any time for the remainder of 2023-2024. Of those self-reporting that they were clinically active 95.7% intended to continue practising medicine in Ireland ($n = 18,496$), while 4.3% did not, despite renewing their registration ($n = 832$).

County of Practise

County of practise was available for 18,799 (97.3%) clinically active doctors in Ireland. The majority of these doctors reported practising in Dublin ($n = 7,903$, 42.0%). A full county breakdown can be observed in the Table 1 below.

Table 1. Doctors' County of Practise

County	n	%
Dublin	7,903	42.0
Cork	2,159	11.5
Galway	1,433	7.6
Limerick	1,026	5.5
Waterford	634	3.4
Louth	525	2.8
Donegal	476	2.5
Kerry	447	2.4
Sligo	423	2.3
Mayo	404	2.1
Kildare	374	2.0
Wexford	339	1.8
Tipperary	330	1.8
Kilkenny	322	1.7
Westmeath	308	1.6
Cavan	284	1.5
Meath	270	1.4
Offaly	254	1.4
Laois	250	1.3
Wicklow	194	1.0
Clare	151	0.8
Roscommon	96	0.5
Carlow	65	0.3
Monaghan	61	0.3
Longford	40	0.2
Leitrim	31	0.2
Total	18,799	100

Note. 529 doctors did not answer this question and are excluded from the above table.

Medical Disciplines and Specialties

Doctors were asked to select which specialty best described their current or most recent practise of medicine. Table 2 shows the number of clinically active doctors in Ireland by the 14 medical disciplines and their corresponding specialties, both as total counts and within the Specialist Division only. In 2023, the highest proportion of clinically active doctors in Ireland reported practising medicine in the specialty of General Practice ($n = 4,990$, 25.8%), followed by General (Internal) Medicine ($n = 1,686$, 8.7%), and Anaesthesiology ($n = 1,265$, 6.5%). Considering the Specialist Division only, the highest proportion of doctors worked in the specialty of General Practice ($n = 3,658$, 37.7%), followed by the Anaesthesiology ($n = 650$, 6.7%) and Psychiatry ($n = 469$, 4.8%).

Table 2. Doctors by Discipline and Specialty (Totals and Specialist Division only)

Medical Discipline/Specialty	Total		Specialist Division	
	n	%	n	%
Anaesthesiology	1,265	6.5	650	6.7
Emergency Medicine	1,019	5.3	200	2.1
General Practice	4,990	25.8	3,658	37.7
Medicine (all 22 Specialties)	4,480	23.2	1,555	16.0
Cardiology	328	1.7	180	1.9
Clinical Genetics	14	0.1	10	0.1
Clinical Neurophysiology	17	0.1	13	0.1
Clinical Pharmacology and Therapeutics	16	0.1	9	0.1
Dermatology	205	1.1	105	1.1
Endocrinology and Diabetes Mellitus	206	1.1	102	1.1
Gastroenterology	270	1.4	155	1.6
General (Internal) Medicine	1,686	8.7	148	1.5
Genito-Urinary Medicine	25	0.1	13	0.1
Geriatric Medicine	397	2.1	150	1.5
Infectious Diseases	104	0.5	50	0.5
Intensive Care Medicine	80	0.4	61	0.6
Medical Oncology	198	1.0	76	0.8
Military Medicine	10	0.1	2	0.0
Nephrology	149	0.8	67	0.7
Neurology	147	0.8	86	0.9
Palliative Medicine	146	0.8	80	0.8
Pharmaceutical Medicine	31	0.2	11	0.1
Rehabilitation Medicine	41	0.2	18	0.2
Respiratory Medicine	266	1.4	138	1.4
Rheumatology	137	0.7	80	0.8
Tropical Medicine	7	0.0	1.0	0.0
Obstetrics and Gynaecology	741	3.8	282	2.9
Occupational Medicine	153	0.8	105	1.1
Ophthalmology	163	0.8	101	1.0
Paediatrics (all 3 Specialties)	1,123	5.8	416	4.3
Neonatology	105	0.5	60	0.6
Paediatric Cardiology	16	0.1	10	0.1
Paediatrics	1,002	5.2	346	3.6
Pathology (all 6 Specialties)	613	3.2	382	3.9
Chemical Pathology	17	0.1	12	0.1
Haematology (Clinical and Laboratory)	197	1.0	105	1.1
Histopathology	237	1.2	159	1.6
Immunology (Clinical and Laboratory)	22	0.1	11	0.1
Microbiology	133	0.7	89	0.9
Neuropathology	7	0.0	6	0.1

<i>Continued</i>	Total		Specialist Division	
	n	%	n	%
Psychiatry (all 4 Specialties)	1,476	7.6	707	7.3
Child and Adolescent Psychiatry	208	1.1	135	1.4
Psychiatry	1,124	5.8	469	4.8
Psychiatry of Learning Disability	45	0.2	33	0.3
Psychiatry of Old Age	99	0.5	70	0.7
Public Health Medicine	293	1.5	173	1.8
Radiology (all 2 Specialties)	664	3.4	460	4.7
Radiation Oncology	97	0.5	50	0.5
Radiology	567	2.9	410	4.2
Sports and Exercise Medicine	28	0.1	25	0.3
Surgery (all 11 Specialties)	2,320	12.0	986	10.2
Cardiothoracic Surgery	99	0.5	40	0.4
General Surgery	815	4.2	251	2.6
Neurosurgery	64	0.3	32	0.3
Ophthalmic Surgery	123	0.6	89	0.9
Oral and Maxillo-Facial Surgery	27	0.1	17	0.2
Otolaryngology	177	0.9	93	1.0
Paediatric Surgery	39	0.2	14	0.1
Plastic, Reconstructive and Aesthetic Surgery	163	0.8	73	0.8
Trauma & Orthopaedic Surgery	533	2.8	233	2.4
Urology	181	0.9	93	1.0
Vascular Surgery	99	0.5	51	0.5

Note. To facilitate comparison of values, columns with percentages are shaded with gradations. The darker the tone, the higher the value within the column.

The proportion of doctors in each division can be observed below. Mirroring the table above, doctors in Sports and Exercise Medicine (89.3%) and General Practice (73.3%) have a good representation of specialists relative to doctors on other divisions. However, only 19.6% of doctors in Emergency Medicine and 34.7% of doctors in Medicine are on the Specialist Division. See Table 3 below for more detail.

Table 3. Proportion of Doctors on each Division by Discipline

Medical Discipline	Trainee Specialist		Supervised		General		Specialist	
	n	%	n	%	n	%	n	%
Anaesthesiology	234	18.5	13	1.0	368	29.1	650	51.4
Emergency Medicine	190	18.6	4	0.4	625	61.3	200	19.6
General Practice	283	5.7	4	0.1	1,045	20.9	3,658	73.3
Medicine	862	19.2	63	1.4	2,000	44.6	1,555	34.7
Obstetrics and Gynaecology	162	21.9	10	1.3	287	38.7	282	38.1
Occupational Medicine	10	6.5	-	0.0	38	24.8	105	68.6
Ophthalmology	29	17.8	3	1.8	30	18.4	101	62.0
Paediatrics	286	25.5	18	1.6	403	35.9	416	37.0
Pathology	123	20.1	1	0.2	107	17.5	382	62.3
Psychiatry	395	26.8	5	0.3	369	25.0	707	47.9
Public Health Medicine	30	10.2	1	0.3	89	30.4	173	59.0
Radiology	133	20.0	-	0.0	71	10.7	460	69.3
Sports and Exercise Medicine	-	0.0	-	0.0	3	10.7	25	89.3
Surgery	368	15.9	32	1.4	934	40.3	986	42.5

Note. To facilitate comparison of values, rows with percentages are shaded with gradations. The darker the tone, the higher the value within the row.

Considering registrants in the Specialist Division only, and among all 14 medical disciplines, Ophthalmology has the highest proportion of doctors closer to typical retirement age, with 52.5% ($n = 53$) aged 55 years or older. These are followed by Occupational Medicine (47.6%, $n = 50$), and Psychiatry (44.0%, $n = 311$). Age statistics for the 14 medical disciplines are outlined in Table 4.

Table 4. Age Statistics for Doctors in the Specialist Division by Medicine Disciplines

Medical Discipline	n	Min. age	Max. age	Mean age	SD	% 55 or older
Anaesthesiology	650	29	74	49.4	9.7	33.1
Emergency Medicine	200	31	77	47.0	9.4	22.0
General Practice	3,658	28	94	48.7	11.4	29.1
Medicine	1,555	32	87	49.1	9.6	28.7
Obstetrics and Gynaecology	282	34	73	50.4	9.6	35.1
Occupational Medicine	105	32	79	53.5	10.3	47.6
Ophthalmology	101	33	86	54.4	11.5	52.5
Paediatrics	416	31	75	48.8	9.0	26.0
Pathology	382	32	81	50.1	9.5	31.4
Psychiatry	707	33	81	52.6	9.8	44.0
Public Health Medicine	173	33	78	51.2	10.9	38.7
Radiology	460	31	75	49.7	9.4	29.6
Sports and Exercise Medicine	25	33	90	50.6	14.5	36.0
Surgery	986	32	82	51.7	9.5	36.7

Note. To facilitate comparison of values, columns with means and percentages are shaded with gradations. The darker the tone, the higher the value within the column.

As it can be observed in Table 5 below, the discipline of Public Health Medicine has the highest proportion of females (77.5%, $n = 134$) and Irish qualifications (92.5%, $n = 160$), and one of the lowest in terms of more than 48 hours worked per week (7.1%, $n = 12$). Moreover, the disciplines with the lowest percentage of females are Surgery (17.5%, $n = 173$) and Sports and Exercise Medicine (16.0%, $n = 4$).

Table 5. Percentage of Female Doctors, Irish Qualifications, and 48+ Working Hours for Doctors in the Specialist Division by Medicine Disciplines

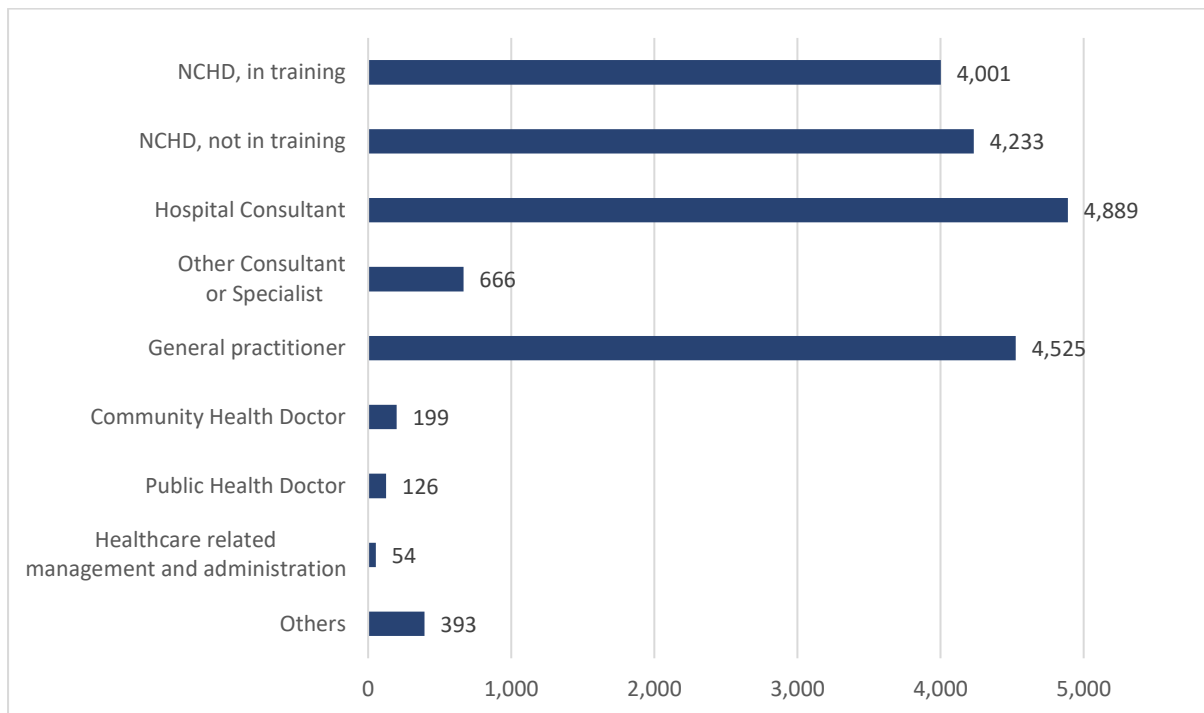
Medical Discipline	n	% Female	% Irish Qualifications	% working More Than 48hs per Week
Anaesthesiology	650	34.0	56.8	23.3
Emergency Medicine	200	25.5	71.0	12.8
General Practice	3,658	55.1	80.2	12.7
Medicine	1,555	43.3	74.0	26.9
Obstetrics and Gynaecology	282	56.4	55.3	33.2
Occupational Medicine	105	58.1	80.0	5.8
Ophthalmology	101	60.4	66.3	2.0
Paediatrics	416	56.7	60.3	26.7
Pathology	382	56.8	76.7	28.1
Psychiatry	707	53.6	74.0	10.0
Public Health Medicine	173	77.5	92.5	7.1
Radiology	460	35.9	73.0	22.7
Sports and Exercise Medicine	25	16.0	76.0	8.7
Surgery	986	17.5	64.1	42.1

Note. To facilitate comparison of values, columns with percentages are shaded with gradations. The darker the tone, the higher the value within the column. 143 doctors did not provide information on hours worked.

Self-Reported Role

Data on self-reported employment role was available for 98.7% ($n = 19,086$) of all clinically active doctors in Ireland. As it can be observed in Figure 7, the most common self-reported employment role among these doctors was hospital consultants ($n = 4,889$, 25.6%), closely followed by GPs ($n = 4,525$, 23.7%).

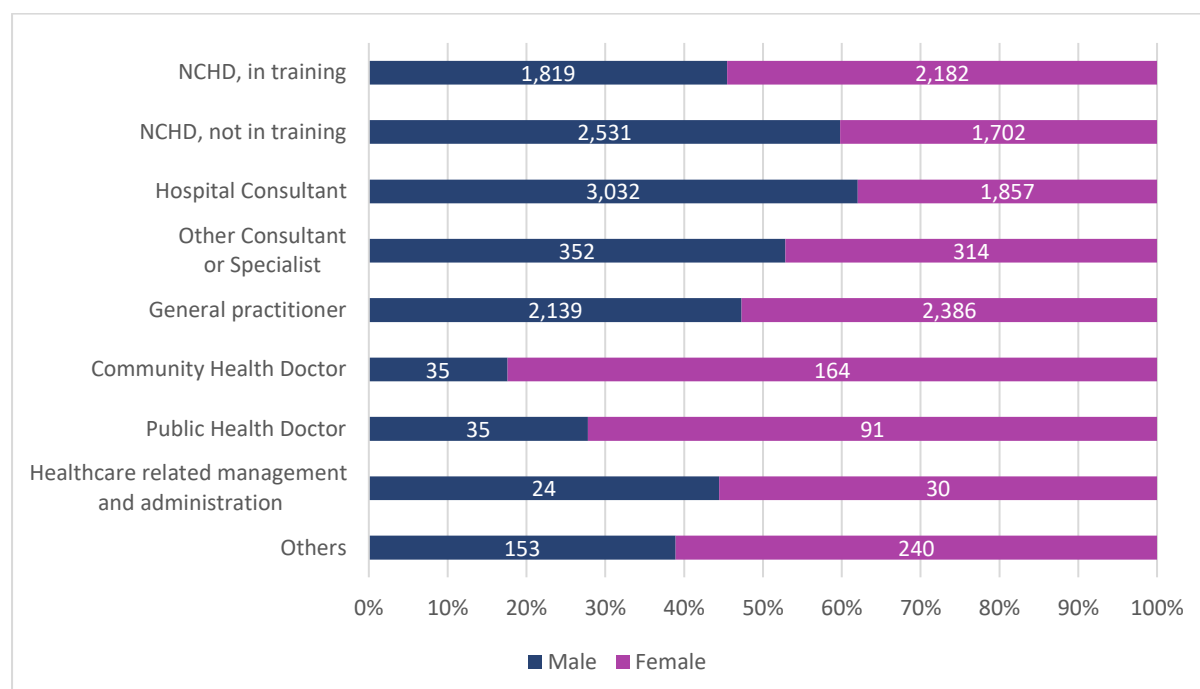
Figure 7. Doctors' Self-Reported Employment Role



Note. 242 doctors did not provide information on self-reported role and are excluded from this figure.

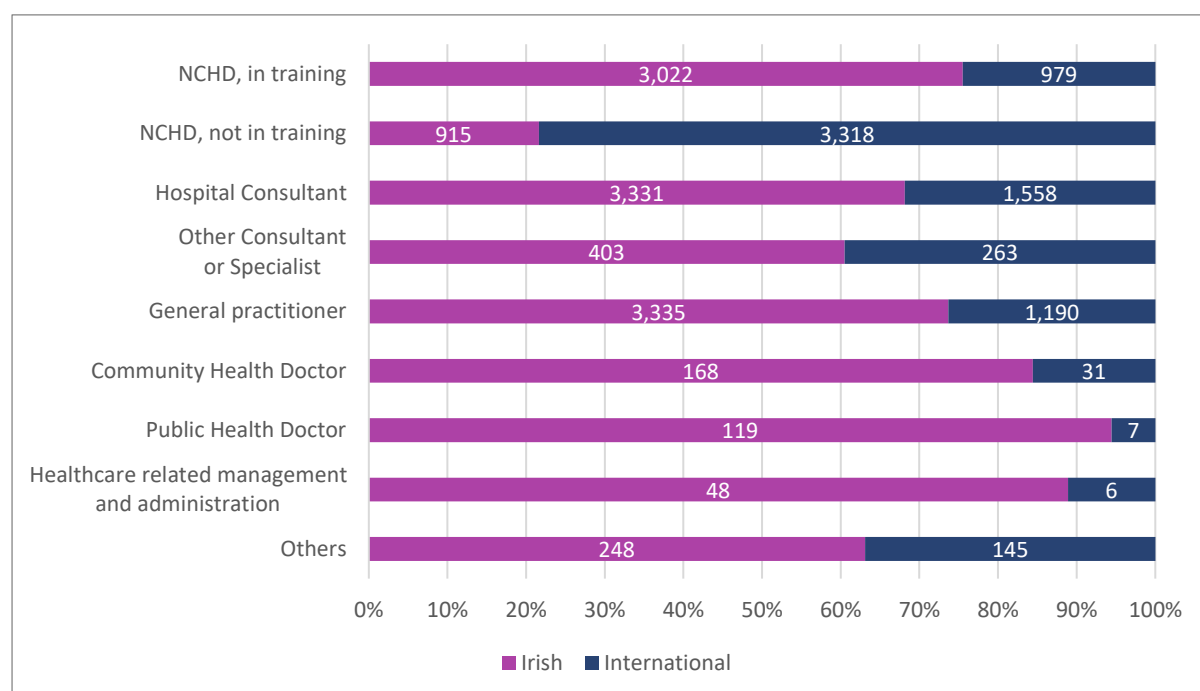
When looking at gender among employment roles, certain roles were closely split among males and females, for example, 52.9% of other consultants or specialists were male and 47.1% were female. Among GPs 47.3% were male and 52.7% were female. However, the proportion of males to females varies for some of the other categories. For example, there was a higher proportion of males than females working as hospital consultants (62% vs 38%) and there was a much higher proportion of females to males working as public health doctors (72.2% vs 27.8%) and community health doctors (82.4% vs 17.6%). There were also slightly more females than males working as a NCHDs in training (54.5% vs 45.5%). Figure 8 shows this in more detail.

Figure 8. Doctors' Self-Reported Role by Gender



In the context of BMQ categories among self-reported employment roles, this translated into over three quarters of NCHDs not in training holding international qualifications ($n = 3,318$, 78.4%) and a similar proportion of NCHDs in training holding Irish qualifications ($n = 3,022$, 75.5%). Figure 9 shows this in more detail.

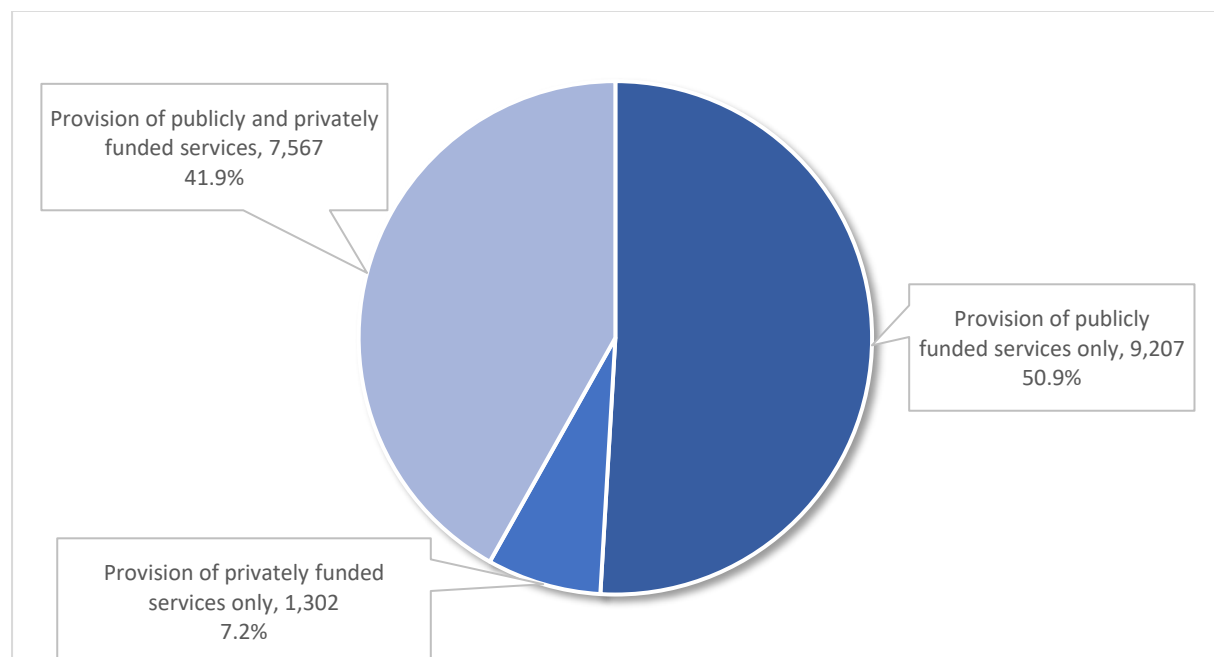
Figure 9. Doctors' Self-Reported Role by Qualification Category



Type of Service Provision

Data on the type of service provided (publicly funded, privately funded, or both) was self-reported by 18,076 (93.5%) clinically active doctors in Ireland. Just over half self-reported providing publicly funded services only ($n = 9,207$, 50.9%), while a further 41.9% ($n = 7,567$) reported providing both publicly and privately funded services, and only 7.2% ($n = 1,302$) reported providing privately funded services only (see Figure 10).

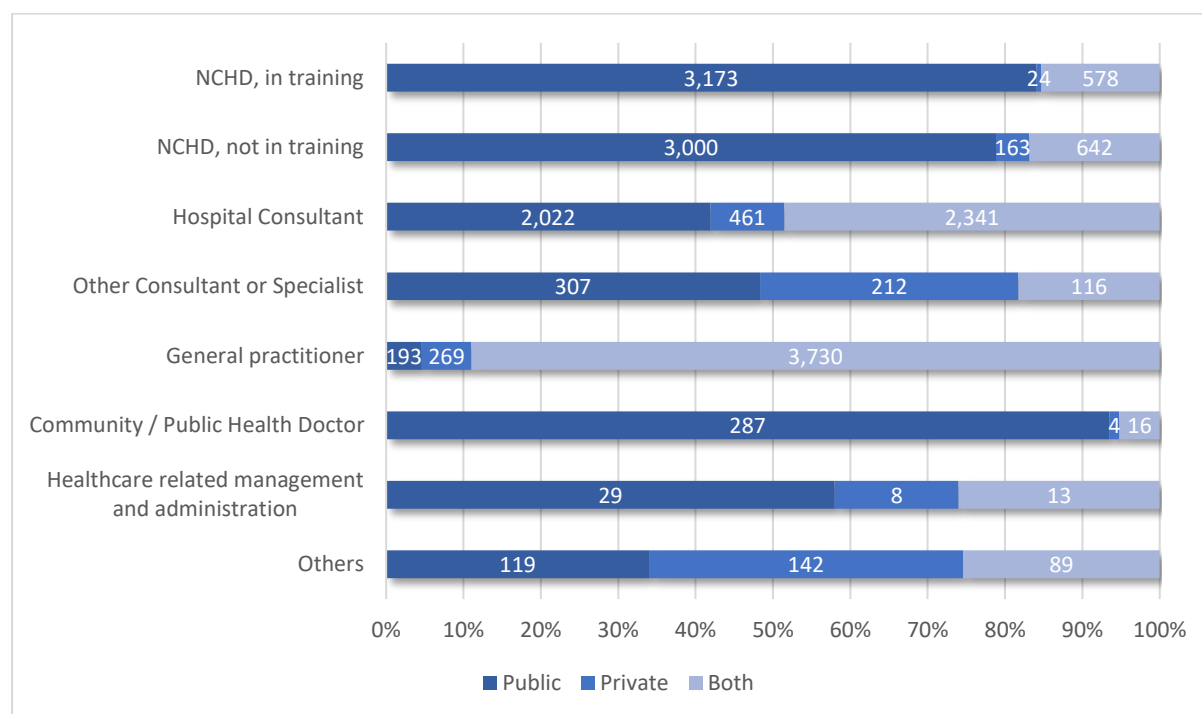
Figure 10. Doctors' Type of Service Provision



Note. 1,252 doctors did not answer this question and are excluded from this figure and the above analysis.

The majority of NCHDs, in training ($n = 3,173$, 84.1%) and not in training ($n = 3,000$, 78.8%) provided publicly funded services only, while the majority of hospital consultants ($n = 2,341$, 48.5%) provided both publicly and privately funded services. See Figure 11 for more detail.

Figure 11. Doctors' Self-Reported Role by Service Provision



Working Hours

Doctors were asked to report the average number of hours they worked per week and 98.4% ($n = 19,025$) doctors provided this detail. Of these, 26.6% ($n = 5,066$) reported working more than 48 hours per week, in contravention of the European Working Time Directive (EWTD). In addition to this, almost a third ($n = 6,202$, 32.6%) reported working 40 to 48 hours per week (Table 6).

Table 6. Doctors' Average Hours Worked per Week

Weekly Hours	n	%
< 10 hours	642	3.4
10 to 20 hours	972	5.1
21 to 30 hours	1,317	6.9
31 to 40 hours	4,826	25.4
40 to 48 hours	6,202	32.6
> 48 hours	5,066	26.6

Note. 303 doctors did not provide information on hours worked and are excluded from this table.

When looking at the percentage of different divisions working in contravention of the EWTD, 45.5% ($n = 1,402$) of all Trainee Specialists reported working more than 48 hours per week. This is in comparison to 26.7% ($n = 1,666$) of those on the General Division, 27.9% ($n = 43$) of those on the Supervised Division and 20.5% ($n = 1,955$) of those on the Specialist Division.

Table 7. Doctors' Average Hours worked per Week by Division

Weekly Hours	Trainee Specialist		Supervised		General		Specialist	
	n	%	n	%	n	%	n	%
< 10 hours	41	1.3	-	0.0	215	3.4	386	4.0
10 to 20 hours	27	0.9	1	0.6	257	4.1	687	7.2
21 to 30 hours	43	1.4	1	0.6	310	5.0	963	10.1
31 to 40 hours	388	12.6	54	35.1	1,624	26.1	2,760	28.9
40 to 48 hours	1,179	38.3	55	35.7	2,162	34.7	2,806	29.4
> 48 hours	1,402	45.5	43	27.9	1,666	26.7	1,955	20.5

Note. 303 doctors did not provide information on hours worked and are excluded from this Table.

A higher proportion of doctors with Irish qualifications ($n = 3,436$, 29.7%) were working more than 48 hours per week in comparison to those with a qualification from outside of Ireland ($n = 1,630$, 21.9%). However, when looking at those working 40-48 hours per week the opposite trend was observed.

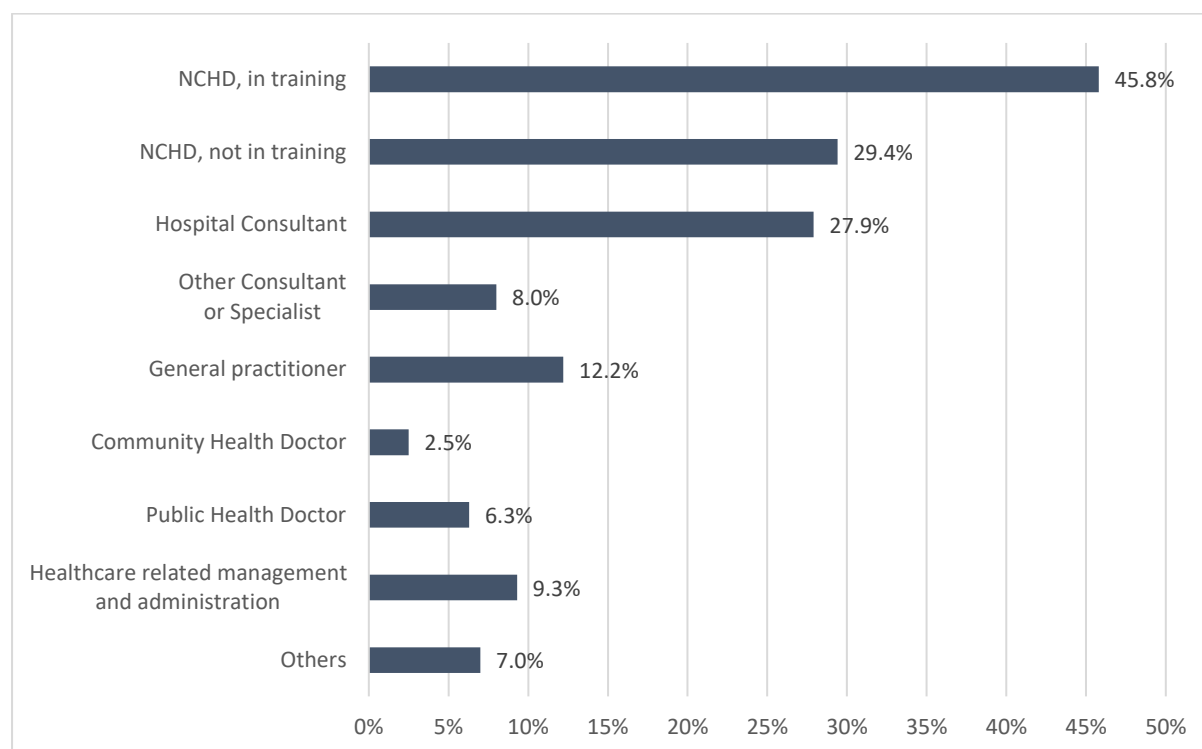
Table 8. Doctors with an Irish or International Qualification by Hours Worked per Week.

Weekly Hours	Irish Qualification		International Qualification	
	n	%	n	%
< 10 hours	472	4.1	170	2.3
10 to 20 hours	741	6.4	231	3.1
21 to 30 hours	925	8.0	392	5.3
31 to 40 hours	2,428	21.0	2,398	32.2
40 to 48 hours	3,572	30.9	2,630	35.3
> 48 hours	3,436	29.7	1,630	21.9

Note. 303 doctors did not provide information on hours worked and are excluded from this Table.

Close to half ($n = 1,825$, 45.8%) of NCHDs in training were working more than 48 hours per week, in contravention with the EWTD. This is in comparison to 29.4% ($n = 1,236$) of NCHDs not in training, 27.9% ($n = 1,351$) of hospital consultants and 12.2% ($n = 547$) of GPs. See Figure 12 below for more detail.

Figure 12. Percentage of Doctors Working in Contravention with EWTB by Self-Reported Role



Note. 360 doctors did not provide information on hours worked or role and are excluded from this figure.

4.3 Spotlight: NCHDs

The Irish healthcare system is heavily reliant on NCHDs to deliver care. In 2023, this cohort comprised of 8,234 doctors. The following results are divided by training status (NCHDs in training and not in training). Just under half of these NCHDs were in training ($n = 4,001$, 48.6%), while just over half self-reported that they were not in training ($n = 4,233$, 51.4%).

As it can be observed in Table 9, the majority of NCHDs in training were female (54.5%, $n = 2,182$), whereas the majority of NCHDs not in training were male (59.8%, $n = 2,531$). Three quarters of NCHDs in training had Irish qualifications, compared to just over one fifth of doctors not in training. From those NCHDs not in training, 26.7% ($n = 1,128$) obtained their qualifications in Pakistan, 22.1% ($n = 935$) in Ireland, and 18.5% ($n = 783$) in Sudan (Table 10).

Table 9. Percentage of Female Doctors, Irish Qualifications, and 48+ Working Hours for NCHDs in Training and not in Training

NCHD Training status	n	% Female	% Irish Qualifications	% Working More Than 48hs Weekly
In training	4,001	54.5	75.5	45.8
Not in training	4,233	40.2	21.6	29.4

Note. 14 doctors in training and 27 not in training did not provide information on hours worked.

Table 10. Country of Qualification for NCHDs in Training and not in Training

NCHD, in training			NCHD, not in training		
Country	n	%	Country	n	%
Ireland	3,045	76.2	Pakistan	1,128	26.7
Pakistan	223	5.6	Ireland	935	22.1
Sudan	111	2.8	Sudan	783	18.5
United Kingdom	107	2.7	Romania	252	6.0
Romania	85	2.1	South Africa	201	4.8
Hungary	67	1.7	Egypt	112	2.6
Poland	53	1.3	India	83	2.0
South Africa	39	1.0	Hungary	55	1.3
Saudi Arabia	29	0.7	Bulgaria	51	1.2
Bulgaria	22	0.6	Nigeria	49	1.2
Slovakia	22	0.6	China	48	1.1
Czech Republic	19	0.5	Libyan Arab Jamahiriya	43	1.0
India	16	0.4	Iraq	40	0.9
Italy	16	0.4	Poland	40	0.9
Bahrain	12	0.3	Czech Republic	36	0.9
Oman	12	0.3	United Kingdom	32	0.8
Nigeria	12	0.3	Malaysia	28	0.7
Egypt	12	0.3	Latvia	24	0.6
Latvia	9	0.2	Slovakia	24	0.6
Rest of the world (35 countries)	86	2.1	Rest of the world (56 countries)	265	5.6

Note. This data was not available at the time of preparing this report for 4 doctors in training and 4 not in training.

When the different areas of practise (medical specialties) are compared among NCHDs in training and not in training, in many cases both groups tend to show higher proportions working in the same specialties (Table 11). However, some differences can be noted. 5.0% ($n = 199$) of NCHDs in training reported working in Emergency Medicine in comparison to 13.6% ($n = 574$) of NCHDs not in training. One in five (20.5%, $n = 866$) NCHDs not in training reported working in General (Internal) Medicine, whereas 14.5% ($n = 579$) of those in training did.

Table 11. Area of Practise of NCHDs in Training and not in Training

NCHD, in training			NCHD, not in training		
	n	%		n	%
Anaesthesiology	314	7.8	Anaesthesiology	348	8.2
Emergency Medicine	199	5.0	Emergency Medicine	574	13.6
General Practice	310	7.7	General Practice	25	0.6
Medicine (all 22 Specialties)	1,296	32.2	Medicine (all 22 Specialties)	1,475	34.8
Cardiology	92	2.3	Cardiology	57	1.3
Clinical Genetics	2	0.0	Clinical Genetics	1	0.0
Clinical Neurophysiology	1	0.0	Clinical Neurophysiology	0	0.0
Clinical Pharmacology and Therapeutics	4	0.1	Clinical Pharmacology and Therapeutics	3	0.1
Dermatology	30	0.7	Dermatology	39	0.9
Endocrinology and Diabetes Mellitus	63	1.6	Endocrinology and Diabetes Mellitus	45	1.1
Gastroenterology	69	1.7	Gastroenterology	49	1.2
General (Internal) Medicine	579	14.5	General (Internal) Medicine	866	20.5
Genito-Urinary Medicine	3	0.1	Genito-Urinary Medicine	4	0.1
Geriatric Medicine	124	3.1	Geriatric Medicine	111	2.6
Infectious Diseases	31	0.8	Infectious Diseases	22	0.5
Intensive Care Medicine	21	0.5	Intensive Care Medicine	10	0.2
Medical Oncology	48	1.2	Medical Oncology	68	1.6
Military Medicine	0	0.0	Military Medicine	0	0.0
Nephrology	36	0.9	Nephrology	41	1.0
Neurology	34	0.8	Neurology	31	0.7
Palliative Medicine	40	1.0	Palliative Medicine	32	0.8
Pharmaceutical Medicine	0	0.0	Pharmaceutical Medicine	1	0.0
Rehabilitation Medicine	6	0.1	Rehabilitation Medicine	17	0.4
Respiratory Medicine	79	2.0	Respiratory Medicine	56	1.3
Rheumatology	34	0.8	Rheumatology	22	0.5
Tropical Medicine	0	0.0	Tropical Medicine	0	0.0
Obstetrics and Gynaecology	224	5.6	Obstetrics and Gynaecology	224	5.3
Occupational Medicine	15	0.4	Occupational Medicine	2	0.0
Ophthalmology	33	0.8	Ophthalmology	29	0.7
Paediatrics (all 3 Specialties)	347	8.6	Paediatrics (all 3 Specialties)	317	7.5
Neonatology	34	0.8	Neonatology	16	0.4
Paediatric Cardiology	5	0.1	Paediatric Cardiology	0	0.0
Paediatric	308	7.7	Paediatrics	301	7.1
Pathology (all 6 Specialties)	147	3.7	Pathology (all 6 Specialties)	114	2.7
Chemical Pathology	4	0.1	Chemical Pathology	0	0.0
Haematology (Clinical and Laboratory)	44	1.1	Haematology (Clinical and Laboratory)	44	1.0
Histopathology	59	1.5	Histopathology	15	0.4
Immunology (Clinical and Laboratory)	7	0.2	Immunology (Clinical and Laboratory)	3	0.1

<i>Continued</i>	NCHD, in training			NCHD, not in training	
	n	%		n	%
Microbiology	32	0.8	Microbiology	13	0.3
Neuropathology	1	0.0	Child and Adolescent Psychiatry	39	0.9
Psychiatry (all 4 Specialties)	453	11.3	Psychiatry (all 4 Specialties)	249	5.9
Child and Adolescent Psychiatry	29	0.7	Child and Adolescent Psychiatry	0	0.0
Psychiatry	395	9.9	Psychiatry	236	5.6
Psychiatry of Learning Disability	8	0.2	Psychiatry of Learning Disability	5	0.1
Psychiatry of Old Age	21	0.5	Psychiatry of Old Age	8	0.2
Public Health Medicine	34	0.8	Public Health Medicine	6	0.1
Radiology (all 2 Specialties)	161	4.0	Radiology (all 2 Specialties)	36	0.9
Radiation Oncology	23	0.6	Radiation Oncology	24	0.6
Radiology	138	3.4	Radiology	12	0.3
Sports and Exercise Medicine	2	0.0	Sports and Exercise Medicine	1	0.0
Surgery (all 11 Specialties)	466	11.4	Surgery (all 11 Specialties)	833	19.7
Cardiothoracic Surgery	21	0.5	Cardiothoracic Surgery	42	1.0
General Surgery	151	3.8	General Surgery	392	9.3
Neurosurgery	12	0.3	Neurosurgery	18	0.4
Ophthalmic Surgery	21	0.5	Ophthalmic Surgery	10	0.2
Oral and Maxillo-Facial Surgery	4	0.1	Oral and Maxillo-Facial Surgery	3	0.1
Otolaryngology	38	0.9	Otolaryngology	47	1.1
Paediatric Surgery	7	0.2	Paediatric Surgery	17	0.4
Plastic, Reconstructive and Aesthetic Surgery	46	1.1	Plastic, Reconstructive and Aesthetic Surgery	24	0.6
Trauma & Orthopaedic Surgery	94	2.3	Trauma & Orthopaedic Surgery	203	4.8
Urology	42	1.0	Urology	51	1.2
Vascular Surgery	30	0.7	Vascular Surgery	26	0.6
Total	4,001	100.0	Total	4,233	100.0

Note. To facilitate comparison of values, columns with percentages are shaded with gradations. The darker the tone, the higher the value within the column.

4.4 Spotlight: Hospital Consultants by Service Provision

This section focuses on clinically active doctors in Ireland who selected hospital consultant as their employment role during the retention process. It offers a comparison by service provision categories: provision of publicly funded services only, provision of privately funded services only, or provision of both publicly and privately funded services. There were 65 doctors who did not answer this question, and therefore analysis is based on 4,824 hospital consultants. Those working in private only had a higher average age (53.3 years) than the other two groups (Figure 13). Moreover, those working in private services had the greatest percentage of doctors aged 55 and older (42.5%) (Figure 14).

Figure 13. Average Age of Hospital Consultants by Service Provision Category

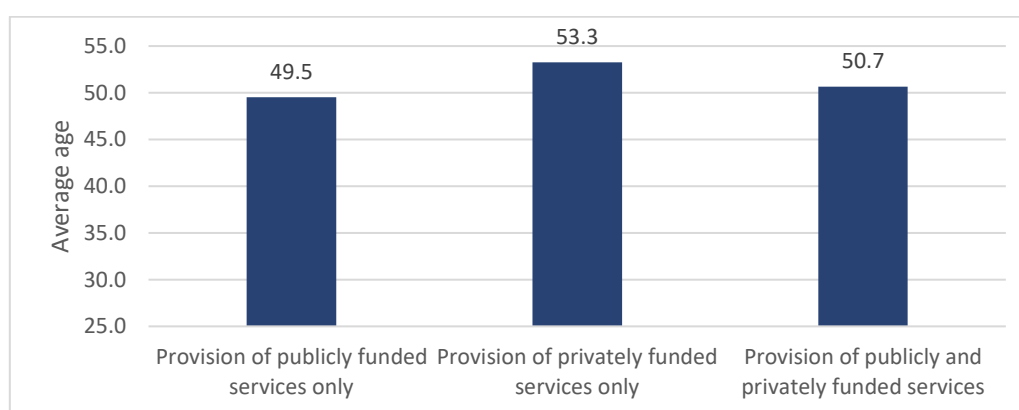
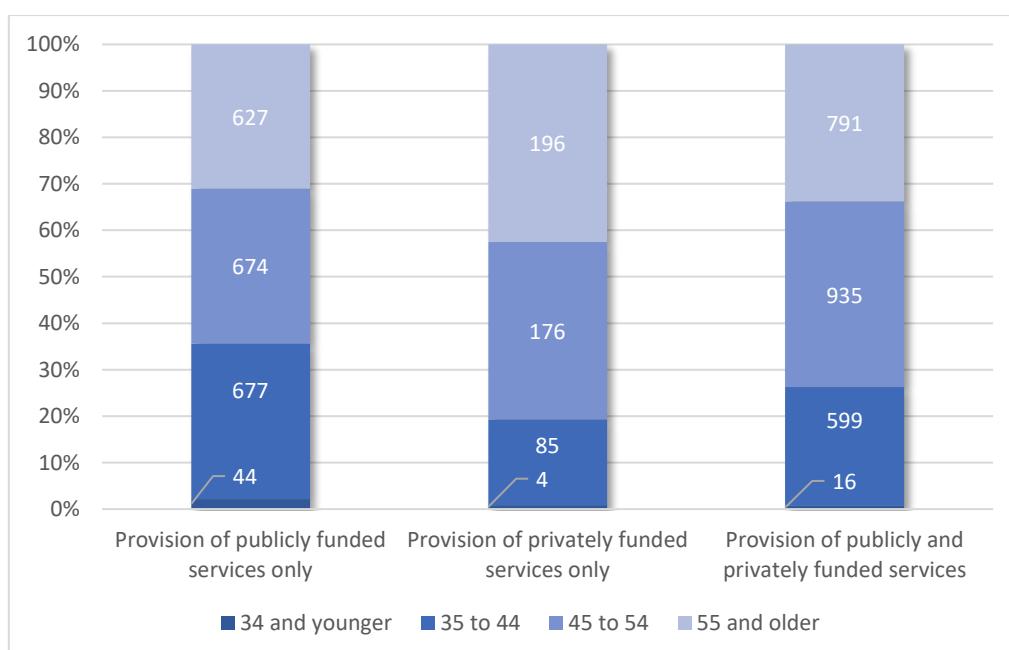
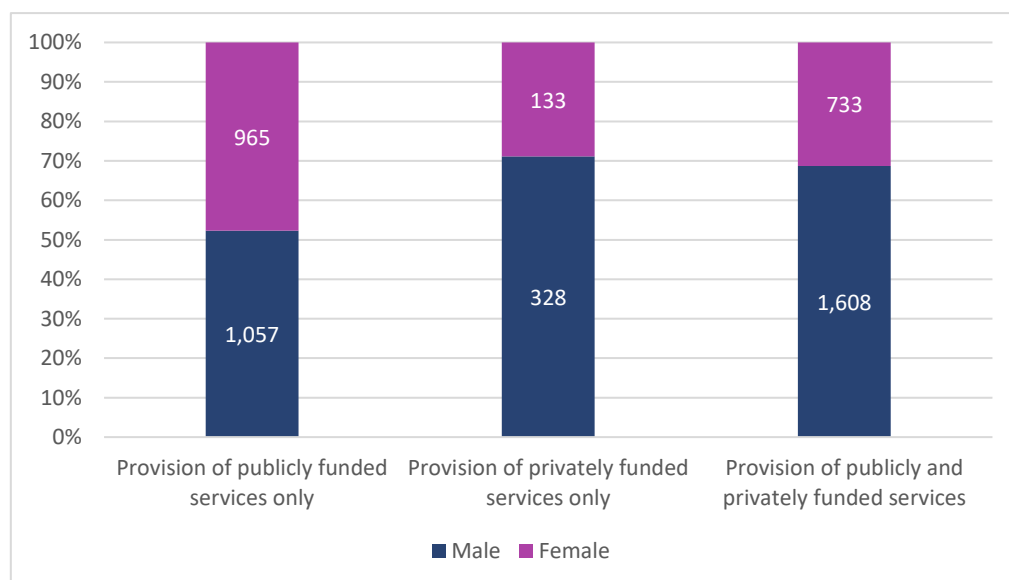


Figure 14. Age Categories of Hospital Consultants by Service Provision Category



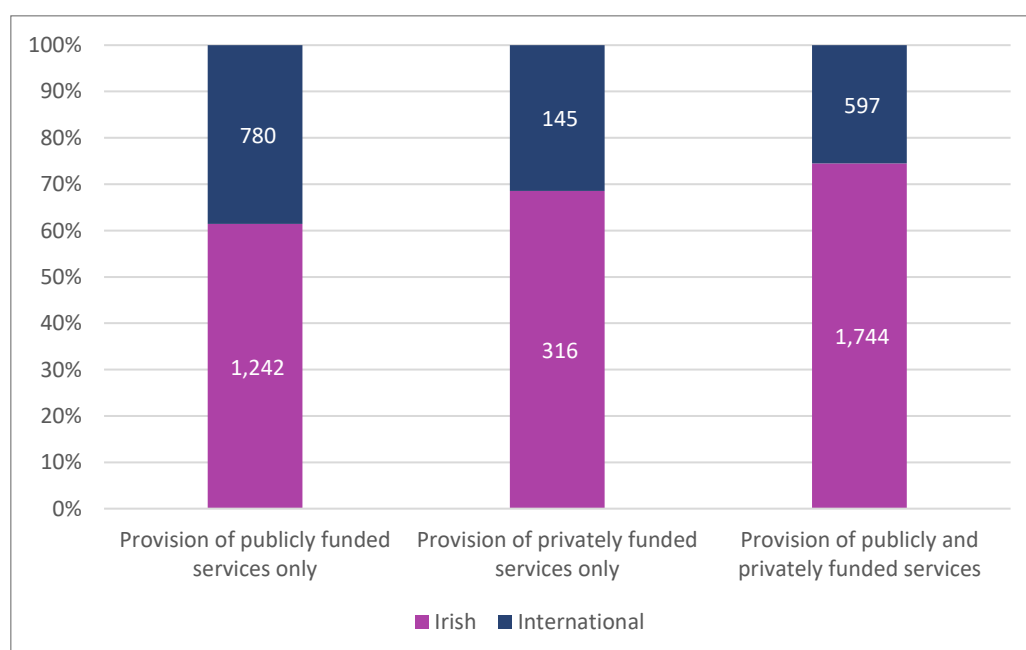
In terms of gender, there was a close to even proportion of males and females among hospital consultants working in publicly funded services only, while in the other two groups there were slightly more than two males for every female (Figure 15).

Figure 15. Gender Breakdown by Service Provision Category



Almost three in four hospital consultants working in services funded both publicly and privately had Irish qualifications (Figure 16).

Figure 16. Qualifications of Hospital Consultants by Service Provision Category



Doctors who worked in both public and privately funded services showed the highest proportion of working more than 48 weekly hours (36.9%, $n = 860$) when compared to those working solely in publicly or privately funded services.

Table 12. Hours Worked per Week by Hospital Consultant by Service Provision Categories

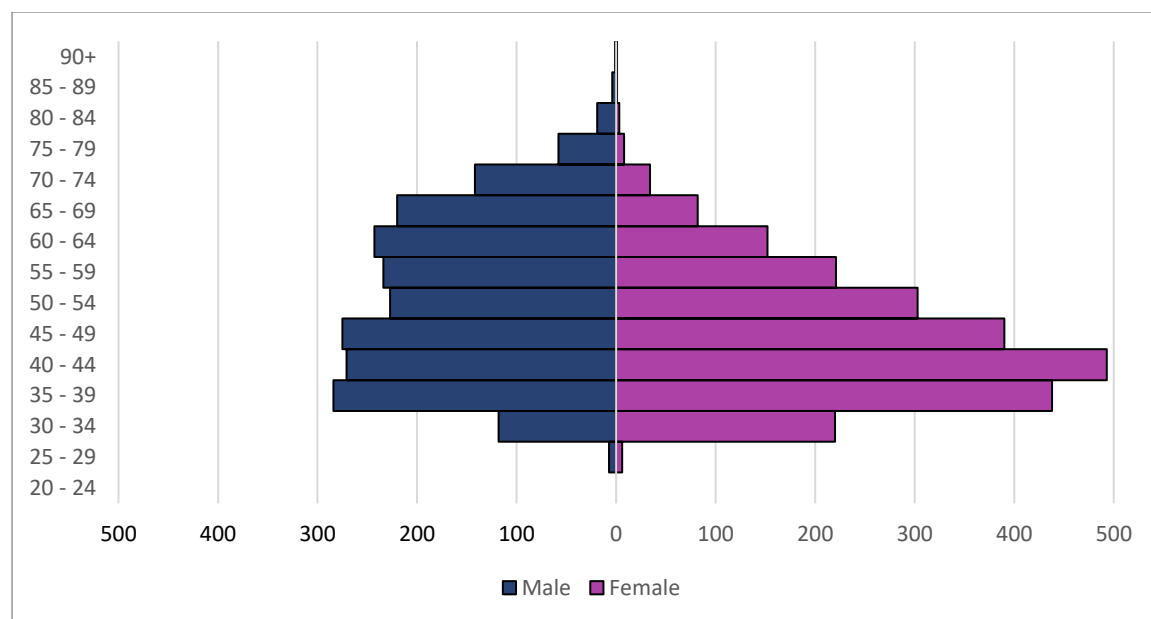
Weekly Hours	Publicly Funded Services Only		Privately Funded Services Only		Publicly and Privately Funded Services	
	n	%	n	%	n	%
Fewer than 10	48	2.4	26	5.8	15	0.6
10 to 20	93	4.6	45	10.0	52	2.2
21 to 30	78	3.9	56	12.4	76	3.3
31 to 40	687	34.3	111	24.6	461	19.8
40 to 48	709	35.4	120	26.5	866	37.2
More than 48	390	19.5	94	20.8	860	36.9
Total	2,005	100.0	452	100.0	2,330	100.0

Note. 102 doctors did not answer one or two of the questions and are excluded from this table.

4.5 Spotlight: General Practitioners Working in General Practice

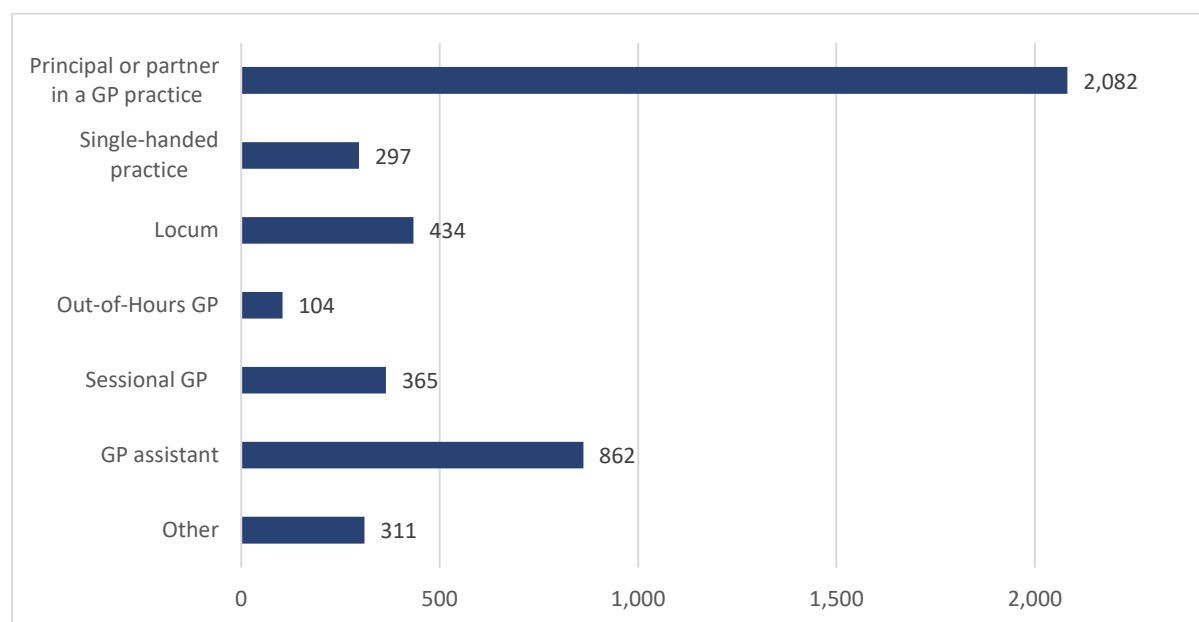
This section focuses exclusively on GPs, that is, doctors who report working in the area of General Practice and whose self-reported employment role is a General Practitioner. This cohort comprised of 4,456 doctors, with just over half being female ($n = 2,353$, 52.8%). The mean age of GPs was 49.3 years ($SD = 11.8$), and 32.0% ($n = 1,424$) were aged 55 years or older, and therefore approaching the typical retirement age. Figure 17 provides a population pyramid of this cohort for more details on gender and age categories composition.

Figure 17. Population Pyramid of General Practitioners Working in General Practice



Starting in 2023, doctors selecting General Practice were asked to specify which type of practice they were working in. Just under half, 46.7% ($n = 2,082$) of GPs reported working as principal or partner in a GP practice, while 19.3% ($n = 862$) worked as GP assistants. Figure 18 provides more details on the breakdown of GP categories.

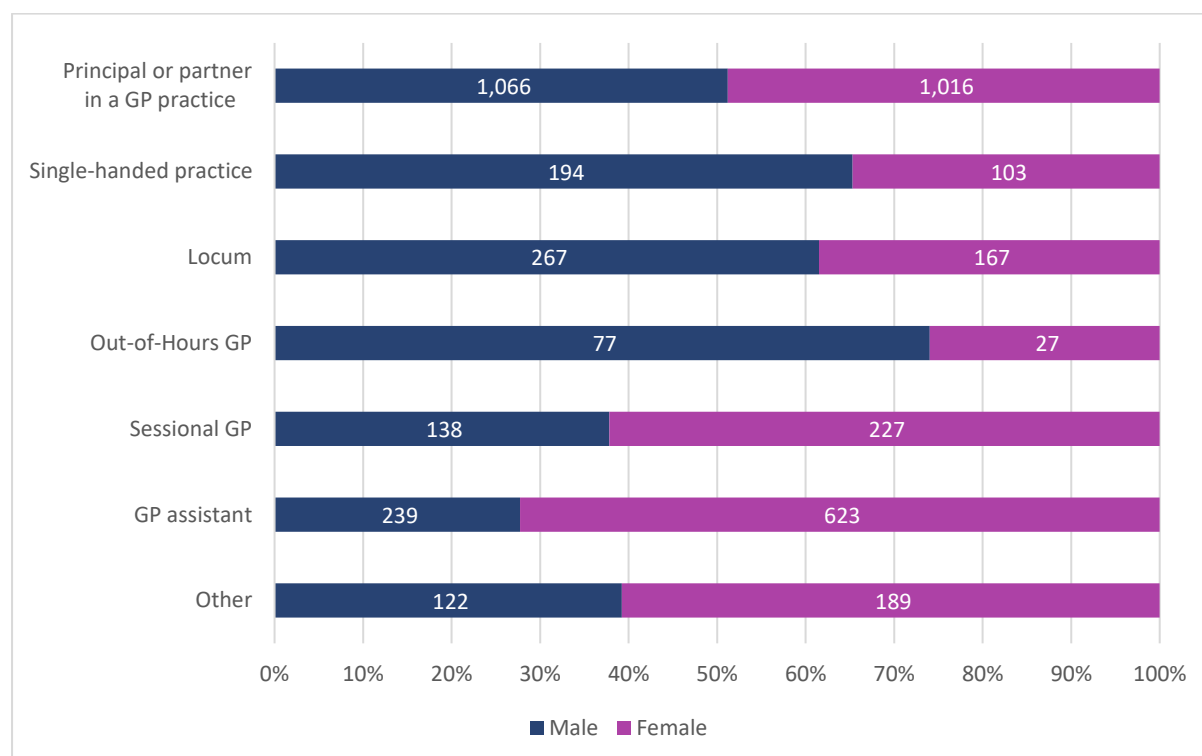
Figure 18. General Practice Categories of General Practitioners Working in General Practice



Note. General practice category is missing for one doctor as the question was not answered.

The breakdown of doctors working as a principal or partner in a GP practice was evenly split, with slightly more females than males. However, there was a higher proportion of males than females in a single-handed practice, and working as a locum or out-of-hours GP. On the other hand, there was a higher proportion of female GPs working as a GP assistant or sessional GP (see Figure 19 below). With the growing proportion of female GPs this breakdown is important to consider.

Figure 19. Gender Breakdown of GP categories



Note. General practice category is missing for one doctor as the question was not answered.

Almost three quarters of GPs (74.0%, $n = 3,299$) held Irish qualifications. In relation to the Divisions of the Register, the Specialist Division encompassed 79.3% ($n = 3,533$) of GPs, followed by the General Division with 19.2% ($n = 856$) and a small proportion of 1.5% ($n = 66$) registered on the Trainee Specialist Division.

Regarding county of practise 1,316 GPs reported practising in County Dublin, accounting for 30.0% of all GPs who provided this information. More details can be observed in Table 13.

Table 13. GPs County of Practise

County	n	%
Dublin	1,316	30.0
Cork	569	13.0
Galway	283	6.5
Limerick	202	4.6
Kildare	163	3.7
Donegal	158	3.6
Kerry	137	3.1
Wicklow	137	3.1
Wexford	136	3.1
Waterford	129	2.9
Tipperary	128	2.9
Meath	118	2.7
Mayo	108	2.5
Louth	106	2.4
Westmeath	94	2.1
Clare	87	2.0
Laois	69	1.6
Sligo	68	1.6
Kilkenny	64	1.5
Cavan	59	1.3
Offaly	56	1.3
Carlow	53	1.2
Roscommon	49	1.1
Monaghan	37	0.8
Longford	36	0.8
Leitrim	24	0.5
Total	4,386	100.0

Note. 70 GPs in General Practice did not provide information on county of practise.

In contravention with the EWTD, 12.3% ($n = 544$) of this cohort reported working more than 48 hours a week, with a further 23.4% ($n = 1,039$) of GPs reporting working between 40 to 48 hours a week.

Table 14. Average Hours Worked per Week among GPs in General Practice

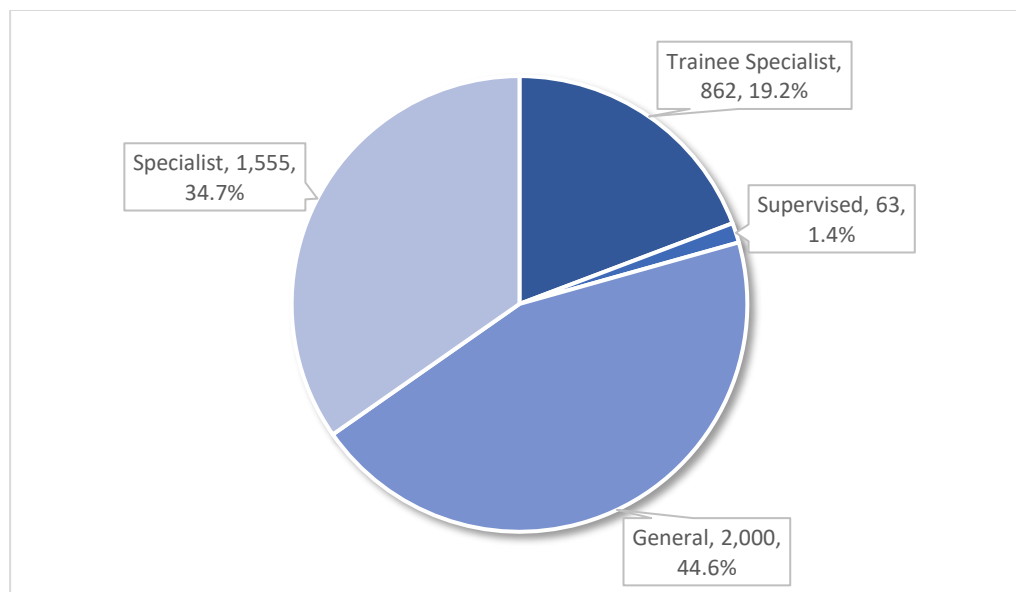
Weekly Hours	n	%
< 10 hours per week	236	5.3
10 to 20 hours	468	10.6
21 to 30 hours	718	16.2
31 to 40 hours	1,426	32.2
40 to 48 hours	1,039	23.4
> 48 hours	544	12.3

Note. 25 GPs in General Practice did not provide information on hours worked.

4.6 Spotlight: Medicine

This section focuses on doctors who work in one of the 22 different areas of practise that fall under the category of medicine (e.g. Cardiology, General (Internal) Medicine). This group comprises 4,480 doctors, 54.0% ($n = 2,419$) of them male. The mean age for doctors in the discipline of medicine was 39.8 (SD = 11.1) years, and 12.9% ($n = 579$) were 55 and older, approaching typical retirement age. The division of the register in which most doctors were registered was the General Division (44.6%, $n = 2,000$) (See Figure 20). Irish qualifications were held by 59.2% ($n = 2,654$) doctors.

Figure 20. Division of the Register for Doctors in Medicine



The following two tables describe doctors practising in the category of medicine who are part of the Specialist Division only, by Medicine discipline specialty. As it can be observed from Table 15, 42.6% ($n = 63$) of doctors working in General (Internal) Medicine currently registered in the Specialist Division are approaching the typical retirement age, along with 29.4% ($n = 53$) of doctors working in Cardiology, 27.3% ($n = 41$) of doctors working in Geriatric Medicine, and 27.1% ($n = 42$) in Gastroenterology.

Table 15. Age Statistics for Doctors in the Specialist Division Working in Medicine Specialties

Medical Discipline Specialty	n	Min. age	Max. age	Mean age	SD	% 55 and older
Cardiology	180	32	83	49.6	10.1	29.4
Clinical Genetics	10	42	63	54.5	8.0	50.0
Clinical Neurophysiology	13	42	87	55.4	12.9	53.8
Clinical Pharmacology and Therapeutics	9	41	66	57.3	8.8	77.8
Dermatology	105	33	73	48.9	9.3	27.6
Endocrinology and Diabetes Mellitus	102	32	82	49.4	9.7	30.4
Gastroenterology	155	32	75	49.5	9.8	27.1
General (Internal) Medicine	148	35	83	52.2	10.4	42.6
Genito-Urinary Medicine	13	33	79	52.2	13.6	38.5
Geriatric Medicine	150	33	74	48.2	9.5	27.3
Infectious Diseases	50	35	76	46.8	9.6	18.0
Intensive Care Medicine	61	32	68	43.4	8.6	13.1
Medical Oncology	76	34	66	47.6	7.9	25.0
Nephrology	67	32	69	48.6	8.5	28.4
Neurology	86	35	79	48.6	8.6	26.7
Palliative Medicine	80	33	70	48.3	8.9	22.5
Pharmaceutical Medicine	11	42	65	53.6	7.5	45.5
Rehabilitation Medicine	18	35	68	50.0	9.6	22.2
Respiratory Medicine	138	32	81	48.3	9.0	23.2
Rheumatology	80	33	69	49.9	9.2	31.3

Note. Military medicine and Tropical medicine have been excluded due to counts lower than 5. To facilitate comparison of values, columns with counts (n), means, and percentages are shaded with gradations. The darker the tone, the higher the value within the column.

As Table 16 illustrates, women constitute a prominent majority in the areas of Palliative Medicine, Dermatology, Clinical Genetics, and Genito-Urinary Medicine with around 70.0% or more female doctors, whereas Cardiology is heavily male dominated, with only 16.7% female doctors. In terms of qualifications, the area of General (Internal) Medicine is the area with the least number of doctors holding Irish qualifications (40.5%, $n = 60$), whereas in Geriatric Medicine, Clinical Neurophysiology, Tropical Medicine, and Clinical Pharmacology and Therapeutics, more than 90% of doctors hold Irish qualifications. Noticeably, the area of practise of Cardiology had the most doctors working more than 48 hours a week, with 46.1% ($n = 83$). This was followed by the areas of Nephrology (35.8%, $n = 24$), and Medical Oncology (34.2%, $n = 26$).

Table 16. Percentage of Female Doctors, Irish Qualifications, and 48+ Working Hours for Doctors in the Specialist Division Working in Medicine Specialties

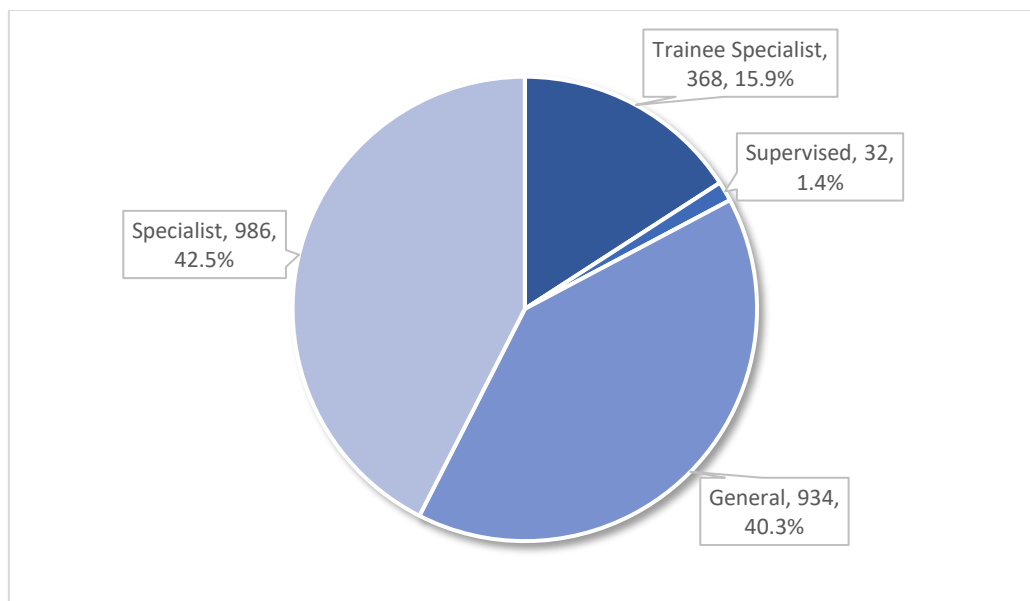
Medical Discipline Specialty	n	% Female	% Irish Qualifications	% Working More Than 48hs Weekly
Cardiology	180	16.7	68.9	46.1
Clinical Genetics	10	70.0	80.0	20.0
Clinical Neurophysiology	13	46.2	92.3	23.1
Clinical Pharmacology and Therapeutics	9	55.6	100.0	33.3
Dermatology	105	74.3	80.0	9.5
Endocrinology and Diabetes Mellitus	102	43.1	72.5	17.6
Gastroenterology	155	33.5	70.3	27.1
General (Internal) Medicine	148	33.1	40.5	23.6
Genito-Urinary Medicine	13	69.2	69.2	0.0
Geriatric Medicine	150	55.3	91.3	28.7
Infectious Diseases	50	56.0	80.0	18.0
Intensive Care Medicine	61	45.9	73.8	21.3
Medical Oncology	76	50.0	77.6	34.2
Nephrology	67	37.3	86.6	35.8
Neurology	86	37.2	69.8	25.6
Palliative Medicine	80	78.8	87.5	23.8
Pharmaceutical Medicine	11	54.5	72.7	9.1
Rehabilitation Medicine	18	66.7	77.8	27.8
Respiratory Medicine	138	30.4	76.8	27.5
Rheumatology	80	45.0	78.8	18.8

Note. Military medicine and Tropical medicine have been excluded due to counts lower than 5. To facilitate comparison of values, columns with counts (n) and percentages are shaded with gradations. The darker the tone, the higher the value within the column. 26 doctors did not provide information on hours worked.

4.7 Spotlight: Surgery

This section concentrates on the cohort of doctors working in one of the 11 specialties under the Medical discipline of Surgery (e.g. Cardiothoracic surgery, Neurosurgery). There were 2,320 doctors in this group, and just over 3 in 4 were male ($n = 1769$, 76.3%). The mean age was 43.0 (SD = 12.1) years and almost one in five ($n = 449$, 19.4%) were 55 or older, and therefore approaching to typical retirement age. There were 986 (42.5%) doctors registered on the Specialist Division (see Figure 21). A slight majority of doctors had Irish qualifications ($n = 1,200$, 51.7%).

Figure 21. Division of the Register of Doctors in Surgery



The remainder of this spotlight section concentrates on doctors in the surgery discipline who are registered in the Specialist Division. From this cohort, as observed in Table 17, 50.0% ($n = 7$) of those working in Paediatric surgery were aged 55 years or older, approaching typical retirement age. The next specialty with the highest proportion of this age cohort was the Oral and Maxillo-Facial surgery with 47.1% ($n = 8$).

Table 17. Age Statistics for Doctors in the Specialist Division working in Surgery Specialties

Surgery Discipline Specialty	n	Min. Age	Max. Age	Mean	SD	% 55 and older
Cardiothoracic Surgery	40	36	64	51.6	7.9	45.0
General Surgery	251	34	81	52.7	9.8	41.4
Neurosurgery	32	40	78	54.3	9.3	37.5
Ophthalmic Surgery	89	32	80	51.1	9.9	34.8
Oral and Maxillo-Facial Surgery	17	40	82	54.5	11.3	47.1
Otolaryngology	93	33	74	52.3	9.6	37.6
Paediatric Surgery	14	38	79	55.1	10.8	50.0
Plastic, Reconstructive and Aesthetic Surgery	73	32	78	50.4	9.4	30.1
Trauma & Orthopaedic Surgery	233	32	80	51.4	9.4	34.8
Urology	93	33	76	50.0	9.5	32.3
Vascular Surgery	51	36	70	50.2	7.7	27.5

Note. To facilitate comparison of values, columns with counts (n), means, and percentages are shaded with gradations. The darker the tone, the higher the value within the column.

Regarding gender, all surgical specialties were male dominated (Table 18). Nonetheless, the three specialties with the greatest female proportion were Ophthalmic Surgery ($n = 28$, 31.5%), Plastic, Reconstructive and Aesthetic Surgery ($n = 23$, 31.5%), and Otolaryngology ($n = 93$, 28.0%). Paediatric surgery was the specialty with the least number of doctors with Irish qualifications ($n = 7$, 50.0%), and conversely, there were 94.1% ($n = 16$) Oral and Maxillo-Facial Surgery doctors with Irish qualifications. Working in excess of 48 hours a week is a strong feature among doctors working in Surgery, especially in Paediatric Surgery ($n = 9$, 64.3%), Vascular Surgery ($n = 28$, 54.9%) and Cardiothoracic Surgery ($n = 20$, 54.1%).

Table 18. Percentage of Female Doctors, Irish Qualifications, and 48+ Working Hours for Doctors in the Specialist Division Working in Surgery Specialties

Surgery Discipline Specialty	n	% Female	% Irish Qualifications	% Working More Than 48hs Weekly
Cardiothoracic Surgery	40	15.0	57.5	54.1
General Surgery	251	17.9	55.4	48.2
Neurosurgery	32	9.4	56.3	50.0
Ophthalmic Surgery	89	31.5	79.8	16.1
Oral and Maxillo-Facial Surgery	17	5.9	94.1	52.9
Otolaryngology	93	28.0	65.6	32.6
Paediatric Surgery	14	14.3	50.0	64.3
Plastic, Reconstructive and Aesthetic Surgery	73	31.5	68.5	33.8
Trauma & Orthopaedic Surgery	233	7.3	66.5	43.2
Urology	93	14.0	66.7	43.3
Vascular Surgery	51	17.6	58.8	54.9

Note. To facilitate comparison of values, columns with counts (n) and percentages are shaded with gradations. The darker the tone, the higher the value within the column. 19 doctors did not provide information on hours worked.

5. Conclusion

Within this report data are analysed and presented to form a comprehensive picture of Ireland's medical workforce. These data are crucial to informing recruitment and retention strategies, medical education and training development and health policy in Ireland. This robust data can support effective planning for a strong and sustainable medical workforce that can provide safe, high quality sustainable patient care into the future.

Significant change is required to ensure NCHDs can work more effectively, efficiently, and safely, delivering the highest standards of patient care. Improving the NCHD experience and work life balance is essential to retaining this critical cohort of doctors. Age of retirement is an important factor in workforce planning, as are doctors' working hours. Excessive work hours are demonstrably associated with attrition, stress, burnout and are predictive of adverse event involvement (Humphries et al., 2021), which can undermine patient safety. The Final Report of the National Taskforce on the NCHD Workforce includes a number of recommendations which seek to address work life balance and the working environment of NCHDs. Each recommendation includes responsible lead organisations and implementation timeframes, and work on implementation is ongoing.

A data driven approach is crucial to develop actionable insights to improve training, education, doctor wellbeing and patient safety. This should be a collaborative and shared effort across all stakeholders, including the Medical Council, HSE/NDTP and DOH to inform the Health and Social Care Workforce Planning Strategy and Action Plan. The Medical Council is committed to supporting and collaborating to develop data and information that support workforce planning, as an integral part of a wider healthcare workforce strategy.

References

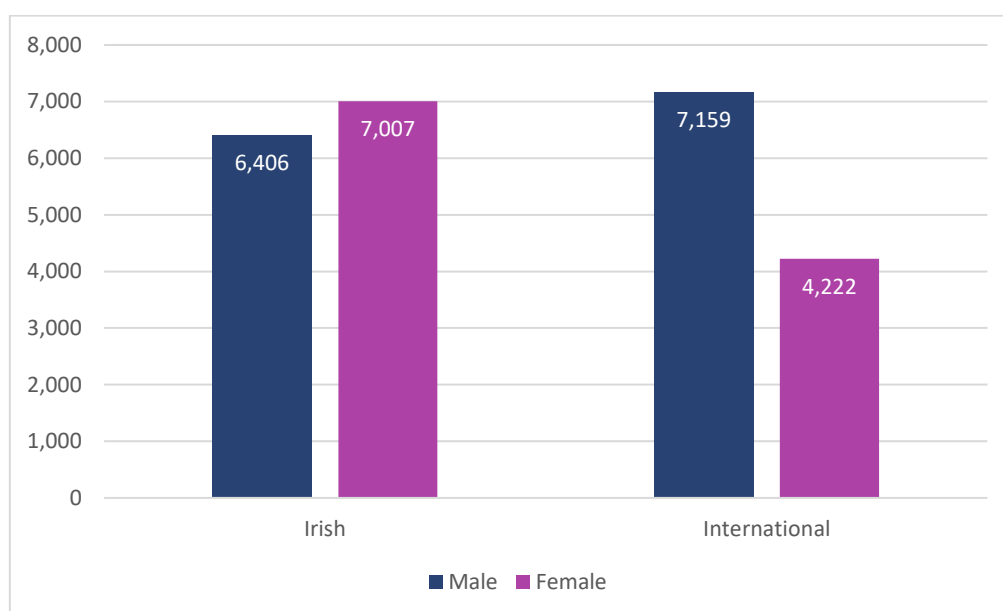
- Collins, C., and O’Riordan, M. (2024) The Future of Irish General Practice: ICGP Member Survey.
Available at: <https://www.lenus.ie/handle/10147/614387>
- Department of Health. (2024). National Taskforce on Non-Consultant Hospital Doctor (NCHD) Workforce: Final Recommendations Report. Available at:
<https://www.gov.ie/en/publication/631e7-national-taskforce-on-the-non-consultant-hospital-doctor-nchd-workforce-final-recommendations-report/>
- Department of Health. (2023). National Taskforce on Non-Consultant Hospital Doctor (NCHD) Workforce: Interim Recommendations Report. Available at:
<https://www.gov.ie/en/publication/73674-national-taskforce-on-the-nchd-workforce-interim-recommendations-report/>
- Heffron, M., & Socha-Dietrich, K. (2019). The Irish paradox: Doctor shortages despite high numbers of domestic and foreign medical graduates. *Recent Trends in International Migration of Doctors, Nurses and Medical Students*. OECD.
- Humphries, N., Creese, J., Byrne, J.-P., & Connell, J. (2021). COVID-19 and doctor emigration: the case of Ireland. *Human resources for health*, 19(1), 1-10.
- Hynes, T., & O’Connor, P. (2023). Spending Review 2022: An Analysis of Medical Workforce Supply. Research Services & Policy Unit, Department of Health, Irish Government Economic and Evaluation Service.
- Medical Workforce Report 2022-2023. (2023). HSE - National Doctors Training and Planning.
Available at: <https://www.hse.ie/eng/staff/leadership-education-development/met/plan/ndtp-medical-workforce-report-2023.pdf>
- National Doctors Training and Planning. (2024). Medical Workforce Analysis Report 2023-2024.
Available at: <https://www.hse.ie/eng/staff/leadership-education-development/met/plan/medical-workforce-report-23-24-digital.pdf>

Appendix

Doctors Offered Retention and Retaining (as a whole)

In June 2023, 25,474 doctors on the Register of Medical Practitioners in Ireland were invited to retain their registration with the Medical Council. Of these doctors, 24,794 (97.3%) chose to retain their registration. Doctors retaining were aged between 24 and 95 years, with an average age of 43.9 years (SD = 12.3). Over half of these doctors were male ($n = 13,565$, 54.7%), and held Irish qualifications ($n = 13,413$, 54.1%). Further detail on gender and qualification breakdown can be observed in Figure A1 below.

Figure A1. Gender Breakdown of Irish and International Doctors Retaining



Just under half of doctors retaining were on the Specialist Division of the Register ($n = 11,621$, 46.9%; see Table A1).

From all doctors retaining, 86.7% ($n = 21,506$) intended to be in medical practise in Ireland at some stage for the remainder of 2023 or 2024, while 13.3% ($n = 3,288$) stated they did not intend to practise. (this provides information on intention, rather than actual practise).

Table A1. *Division of Doctors Retaining*

	n	%
Trainee Specialist Division	3,271	13.2
Supervised Division	177	0.7
General Division	9,725	39.2
Specialist Division	11,621	46.9
Total	24,794	100.0

Glossary

C

Clinically Active (Doctors): All doctors who retain their place on the register are asked if they have practised medicine in the past 12 months. If a doctor selects 'yes' to this question they are defined as being clinically active.

Clinically Active in Ireland: All doctors reporting to be clinically active over the past 12 months, are asked where they practised medicine. If they reported practising in the Republic of Ireland only, or both within and outside the Republic of Ireland they are defined as being clinically active in Ireland.

D

Doctors Retaining: All doctors who choose to renew their registration. Doctors retain their registration by completing the annual retention process and paying the required fee.

I

International Graduates: Doctors with a basic medical qualification from outside of Ireland.

International Qualification: Basic medical qualification from outside of Ireland.

Irish Graduates: Doctors with a basic medical qualification from Ireland.

Irish Qualification: Basic medical qualification from Ireland.

N

Not Practising: All doctors who retain their place on the register are asked if they practised medicine in the past 12 months. If doctors selects 'no' to this question they are not clinically active and defined as not practising.

S

Standard Deviation (SD): statistic measuring how spread the data is around the mean.

O

Offered Retention: All doctors who are active on the register must have their registration renewed, on expiry of a Certificate of Registration, which will occur at least once every twelve months. Offered retention therefore covers all doctors active on the register who are invited to retain their place on the register.

Q

Qualification Categories: This refers to a doctors' basic medical qualification, and is grouped as doctors with an Irish qualification, doctors with a qualification from either the EU or UK, or doctors with a qualification from outside of Ireland, the EU or the UK.

Abbreviations

ARAF	Annual Retention Application Form
BMQ	Basic Medical Qualification
BST	Basic Specialist Training
CSCST	Certificate of Satisfactory Completion of Specialist Training
DOH	Department of Health
EEA	European Economic Area
EU	European Union
EWTD	European Working Time Directive
GP	General Practitioner
HSE	Health Service Executive
HST	Higher Specialist Training
IMG	International Medical Graduate
IMGTI	International Medical Graduate Training Initiative
NCHD	Non-Consultant Hospital Doctor
NDTP	National Doctors Training and Planning
PGTB	Post Graduate Training Body
SD	Standard Deviation
UK	United Kingdom